NIHR | National Institute for Health and Care Research

Summary

Reference Number	37841
Lead Applicant	Miss Victoria Mummery
Research Title	A qualitative explorative study to understand perspectives on consultant nurse and consultant practitioner roles in UK urgent and emergency care settings to inform the development of a toolkit for role introduction, implementation and impact
Plain English Summary	**Written with support and review of the NIHR Research Support Service PPIE team** A study to find out how consultant nurse and consultant practitioner roles are started and then supported in urgent and emergency care (UEC) services so that a toolkit can be produced together, by staff, researchers and patients, to improve services in the future. **Background:** UEC includes NHS services for both life-threatening and non-life-threatening injuries and illnesses. These services are provided in an Emergency Department (ED) or Urgent Treatment Centre. As the UK population grows and lives longer, more people need to use these services. This means we need to think about the people who work there, and make sure they have the right skills to look after the population. Consultant practitioners (CP) include highly experienced skilled nurses or allied health professionals. Each ED is meant to have at least one CP, however a recent survey found this was not the case, and we don't know why. This means not everyone may get the same care, with some EDs having more senior staff than others. Across the healthcare system CPs have been good, helping to make patients better and happier when unwell. **What do we want to do?** To find out: 1. What it's like to be a CP, especially when starting as a CP, how they make a difference in UK UEC settings, as well as to find out from service-users and NHS employers. 2. What CPs think about the good and bad of starting in this role, if they feel looked after and prepared in the role, and what they would like done differently. 3. If patients like having CPs and if they think they are helpful or what would be helpful for CP roles of the future. Together with patients and CPs, we want to create a toolkit to find out how to get CPs into all UEC services, and then look at how much good they are doing, or they can do to improve services in the future. **How are we going to do it:** There will be four parts: 1. We will look at published research on this topic 2. We will interview CPs, NHS staff who employ CPs and individuals working in integrated care boards who help put together workforce strategies across the UK 3. We will have group discussions with NHS staff in CP roles in UEC and patients/service-users who have recently accessed UEC across the UK 4. After this we will run a workshop to co-produce a toolkit to help bring in and see how good CPs are, ready to be tested in the workplace. **Public involvement and engagement:** Patients/service-users who recently accessed UEC gave their thoughts and ideas; they felt this was an important research idea because it is hard working in UEC, and there's not always enough or the right people to help patients. A study patient and public advisory group (PAG) will be set up. Group members will be involved in developing materials for getting people involved in the study and coming up with questions for interviews and group discussions. They will help look at findings, put together workshop documents and share all study findings with the right people. The group will meet three times in the study, and in between, researchers will share updates through email or letters. **Sharing the findings:** We will share findings in research journals, talk at local and national conferences. We will also talk about our findings with policymakers and professional bodies so together we can come up with the next steps, as well as test our new toolkit.

Pre-submission

Our purpose for collecting this data is to communicate with you about your application and have the necessary information to evaluate you for a grant. The data we collect here is collected in the public interest. Information provided here may be subject to Freedom of Information requests.

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The NIHR privacy policy includes further information including ways we may use your data, our contact details and details on your individual rights regarding how your data is used. Your data may be shared across the NIHR, including with other coordinating centres, to allow the application to be managed and for statistical analysis, and with external grant reviewers as part of the process for managing the allocation of a grant. This notice is under constant review and will be updated and / or revised based on that review as appropriate.

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1. Application Summary Information

Host organisation (which will administer any award)
St George's University Hospitals NHS Foundation Trust

Partner organisation	City St George's, University of London

Research Title	A qualitative explorative study to understand perspectives on consultant nurse and consultant practitioner roles in UK urgent and emergency care settings to inform the development of a toolkit for role introduction, implementation and impact

Proposed WTE Please select which WTE option you would prefer
1.00

Duration (months) The duration for your award is based on your chosen WTE
36

Proposed start date if grant awarded	01/09/2025

Research Type	Qualitative study

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2. Applicant CV

A CV for the Applicant is a mandatory component of this application; please ensure this is completed fully.
Please note that some of the CV questions are automatically completed from information you have provided in the 'Update CV' section of the 'Manage My Details' page. These can be updated via the ARAMIS Home Page or by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Degrees and Professional Qualifications

From	To	Qualification	Subject	Country	Institution	Class
04/2024	07/2024	30 credit module	PhD Funding Proposal Development (30 credit module - compulsory module for pre-doctoral bridging fellowship)	United Kingdom (England)	King's College London	Pass
09/2021	04/2024	MSc	Masters in Clinical Research	United Kingdom (England)	St George's University of London	Merit
09/2019	07/2020	PGCert	Healthcare Research Skills and Methods	United Kingdom (England)	St George's University of London	Pass
04/2016	11/2016	PGCert	Independent Prescribing for Non-Medical Practitioners	United Kingdom (England)	London Southbank University	Pass
01/2016	04/2016	PGCert	Advanced Assessment for non-medical practitioners	United Kingdom (England)	King's College London	Pass
09/2009	06/2012	BSc	Physiotherapy	United Kingdom (England)	St George's University of London	2:1

Present and previous positions
(including from/to dates, job title and employer)

From	To	Position	Organisation	WTE % Research
07/2023	-	Clinical Research Fellow	St George's University Hospital NHS Foundation Trust	100
11/2021	-	Lead Emergency Practitioner	St George's University Hospital NHS Foundation Trust	20
09/2021	01/2023	Teaching Fellow	University of Surrey	0
12/2017	11/2021	Emergency Practitioner	St George's University Hospital NHS Foundation Trust	0
12/2016	12/2017	CLAHRC NWL Project Manager	Imperial College London Hospital/CLAHRC NWL	100
01/2014	12/2016	Specialised Physiotherapist	St George's University Hospital NHS Foundation Trust	0
08/2012	01/2014	Rotational Physiotherapist	St George's University Hospital NHS Foundation Trust	0
08/2011	08/2012	Physiotherapy Technician	St George's University Hospital NHS Foundation Trust	0

Research Grants held

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Title of Grant	Role in the Research Grant	Total Grant £	Source of Grant (Organisation Name)	Duration (In Months)	Start Date of Funding	End Date of Funding
RCEM Research Grants - ConNur-ED Study	Co-Applicant and member of study panel - roles and responsibilities include study and data co-ordinator, data analysis and write-up and presentation at RCEM conference	6350	Royal College of Emergency Medicine	12	01/01/2024	01/12/2024

Publication Record

This should display the authors, date, article title and journal name for each publication added. This information can be edited via the “My Research Outputs” page of your ARAMIS account.

Publications
Mummery, V., Rogers, E., Padmanaban, V., Matthew, D., Woodcock, T. and Bloch, S.. (2019) ‘Transcutaneous carbon dioxide measurement is not a reliable alternative to arterial blood gas sampling in the acute medical setting.’, *The European respiratory journal*, 53(4).

Relevant Prizes, Awards and other Academic Distinctions

Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
01/11/2024	NIHR Emergency Care Incubator	Successful application for bursary for a fully funded place to attend the NIHR Emergency Care Incubator Course	Attendee
01/01/2024	King's College London/LSBU	HEE ICA Pre-Doctoral Bridging Fellowship - successful applicant. One year fellowship	Fellowship Applicant
01/09/2017	CLAHRC NWL	Brian Turley Award for PPI for project undertaken collaboratively between Imperial College London Hospital and CLAHRC NWL	Project Manager

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ORCID iD 0009-0006-6666-2981

3. Applicant Research Background

Professional Background	Allied Health Professional

Profession (Allied Health Professional)	Physiotherapist

Do you have current / active professional registration? This registration must be with one of the **ICA approved regulatory bodies**
Yes

Professional registration number/pin Please supply a valid professional registration number/pin followed by the regulatory body (eg 12A3456B - NMC or PA12345 - HCPC)
PH99515

Please describe your research career to date

Building on 12-years of research, clinical, and leadership experience, I am ready to accelerate development as a clinical academic and future leader through this fellowship. This fellowship will support my progression towards my goal of becoming a post-doctoral clinical academic practitioner in Urgent and Emergency Care (UEC) with a physiotherapy background.

Research and academic experience:

My passion for research began with my undergraduate dissertation and has continued throughout my career. As a newly qualified physiotherapist, I identified a gap in both my role and job satisfaction. This prompted me to seek research opportunities, leading to appointment as project manager for NIHR NWL Collaboration for Leadership in Applied Health Research (CLAHRC). In this role, I led a year-long quality improvement (QI) project on non-invasive ventilation and an interventional trial on novel monitoring techniques for acute respiratory failure.

On return to full-time clinical practice in UEC, I continued developing my research interests. In 2020, I submitted an NIHR PCAF application focusing on UEC workforce, assessed as 'fundable but not funded,' with feedback highlighting my strong commitment to a clinical academic career. Although unsuccessful, I continued pursuing research development alongside my clinical role by completing a PGCert in Research Methods. I progressed this to complete an MRes (2024), exploring impact of enhanced and advanced practice in UEC. My research interests in UEC workforce align with my advancing practice role and national priorities for UEC transformation, alongside my MRes study topic.

To build this topic toward a DCAF I was successfully awarded a pre-doctoral bridging fellowship, supporting development of this application. This fellowship provided a platform to advance my skills and knowledge as an aspiring clinical academic and supported identified learning gaps including research/grant funding. The scheme has enhanced my awareness of supervisory relationships, including understanding supervisory roles and complementarity of expertise in developing strong research applications. It also allowed me to build my supervisory and mentorship team for the DCAF.

I have actively pursued workshops, webinars, and academic training relevant to my clinical setting, alongside membership in the NIHR Emergency Care Incubator, and participation in an RSS-led fellowship application day and NIHR PI essentials training. I have led service evaluations and audits, including recent projects on ankle sprain and ambulance access to UEC. To broaden my clinical studies experience, I was associate Principal Investigator for a national study in management of fractures (DRAFT-CASP) and am currently Principal Investigator for a national study in cellulitis management. I am contributing to an expert panel on an NIHR-funded study exploring UEC non-medical practitioner roles (SkillMix-ED NIHR131356). I am a co-applicant on a Royal College of Emergency Medicine (RCEM) grant-funded study on UEC consultant-level practice roles, which further inspired my DCAF project. I was appointed as a research fellow one-day/week on this study supporting initial grant writing, leading a scoping review around UEC workforce roles, and disseminating findings. This dual role as

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clinician and research fellow has proved pivotal in immersing myself in the field of UEC clinical academia.

My career experiences include but are not limited to curation, analysis, methodology and project administration, alongside sound research governance. My outputs include publications in peer-reviewed journals, conference presentations, and NICE best practice and British Thoracic Society guidance contributions.

*2016 – **Co-author** paper - Physiotherapy Journal https://doi.org/10.1016/j.physio.2016.10.262*

*2016 – **Co-author** paper - European Respiratory Journal http://dx.doi.org/10.1183/13993003.congress-2016.PA4420*

*2017 – **Reviewer** - NCEPOD 'Acute Non-Invasive Ventilation: Inspiring Change' NCEPOD - Non-Invasive Ventilation: Inspiring Change (2017)*

*2018 – **Contributor** - British Thoracic Society Non-Invasive Ventilation QI Tool Non-Invasive Ventilation | British Thoracic Society | Better lung health for all (brit-thoracic.org.uk)*

*2018 – **Contributor** - NICE Best Practice Guidance INTU (Improving Non-invasive ventilation through understanding) project*

*2018 – **Lead author** 2 x papers - BMJ Thorax https://doi.org/10.1136/thorax-2018-212555.352 http://dx.doi.org/10.1136/thorax-2018-212555.350*

2018-2019 – Oral/poster presentations INTU project at national conferences

*2024 – **Co-author** abstract, oral presentation, poster - RCEM https://doi.org/10.1136/emermed-2024-RCEM.39,*

*2024 – **Lead author** 2 x posters City St George's Research Day*

Profession and clinical experience:

Over 12-years, my career has advanced into a senior leadership role in a large UEC multi-professional team. As a respected member of the leadership team, I lead service improvement projects, develop clinical guidelines, and supervise early career colleagues, while strengthening my expertise in advanced clinical practice. As the first physiotherapist in a clinical leadership role within my UEC department I've pioneered integration of physiotherapy practitioners into the team, advocating for the discipline through recruitment, pathway development, and advancing autonomy. I've shared this work through networks, for example Chartered Society of Physiotherapy (CSP) and Advanced Practice Physiotherapy Network (APPN).

Education and teaching development:

I actively seek opportunities to share knowledge, supervising staff and students while managing twenty-one practitioners. In addition to internal teaching, I contribute to undergraduate and postgraduate education and contribute as an OSCE invigilator for advanced practice modules. I was a teaching fellow in advanced assessment at Surrey University (2021-2022). My interest in QI led me to complete a QI coaching programme (2017), enabling me to coach others through QI projects.

Research leadership:

My research leadership skills, particularly in governance, PPIE, and study setup, significantly developed during my CLAHRC role and have continued to grow in my current research fellow position. Leadership courses for my clinical role have provided transferrable skills for academic leadership, enabling me to supervise research nurses in publication writing. I maintain Good Clinical Practice compliance and have completed introductory PPIE training to enhance my involvement in a hospital patient engagement group.

Career trajectory:

I have advanced in my clinical role and am a respected leader within my specialty, continuously expanding my knowledge and skills to deliver high-quality care. I am eager to advance my clinical academic career through the NIHR schemes, with the DCAF as the next step. I aim to mentor and supervise UEC staff, role modelling research development, and collaborate with service-users to strengthen existing research culture in UEC. I aim to strengthen my relationship with the partner HEI for an honorary contract and use this fellowship to explore post-doctoral opportunities with NIHR, advancing my career and networking within the clinical academic community.

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Have you already registered for a PhD?	No

Has this application, or a similar application, previously been submitted to this or any other funding body?
No

Contextual Factors
Please use this question to detail any contextual factors you wish to make the Selection Committee aware of. NIHR wants to know about any circumstances so that they may take them into consideration during the assessment of your application. Contextual factors may include:

- Career breaks due to parental leave, or periods of illness.
- Reduced time spent undertaking research due to a disability or caring responsibilities. This could include any physical or mental difficulty that may have impacted your research career. These are situations that have a significant impact on your ability to undertake research.
- Reduced opportunities to career support e.g. mentorship, and limited opportunities to undertake prior research and training.
- Impact of the COVID-19 pandemic on your research career.

Please also use this section to detail any other factors that may have impacted your research career not listed in the examples provided. The impact on your career to date will be specific to your particular circumstances but could include such impacts as limited opportunities to obtain grant funding or fewer publications. In general terms, mitigating factors should be significant, and relevant.

NIHR acknowledges that you may be reluctant, or uncomfortable disclosing relevant information that is sensitive. However, you should bear in mind that we are unable to take into account factors that you do not disclose. Please be assured that the information provided by you is sensitive and will be treated confidentially and in line with General Data and Protection Regulations (GDPR).

- Since undertaking further study and transitioning into my practitioner role 6years ago, I have had to prioritise my clinical development to ensure my competence and confidence in my advancing practice role. Before starting my practitioner role in ED, there were limited research development and mentorship opportunities within my previous team. This is one of the reasons for undertaking the one-year CLAHRC role to seek such opportunities that prior appeared unavailable.

- Whilst my current role should entail all elements of advancing practice, due to operational, managerial and leadership demands often research and education time has been limited and been required to fit around operational and managerial responsibilities. The short-term ED research fellow has allowed me to balance the passions of my senior clinical leadership role with research and academia with the support of Professor Heather Jarman. Since these opportunities have arisen in the last year, I have developed immensely regarding contribution to grant applications and publications.

- Working currently in a busy UEC service has resulted in a demanding senior leadership role in ED. Over the past three years I have developed my team and been succession planning to enable me to fulfil my dream to undertake a full-time DCAF. This planning will allow me to step away from the demands of my current role, with the 20% clinical time ensuring I continue to maintain my advanced clinical skills alongside developing the skills that are required for the next step in my professional leadership journey.

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4. Plain English Summary of Research

Written with support and review of the NIHR Research Support Service PPIE team

A study to find out how consultant nurse and consultant practitioner roles are started and then supported in urgent and emergency care (UEC) services so that a toolkit can be produced together, by staff, researchers and patients, to improve services in the future.

Background:

UEC includes NHS services for both life-threatening and non-life-threatening injuries and illnesses. These services are provided in an Emergency Department (ED) or Urgent Treatment Centre. As the UK population grows and lives longer, more people need to use these services. This means we need to think about the people who work there, and make sure they have the right skills to look after the population.

Consultant practitioners (CP) include highly experienced skilled nurses or allied health professionals. Each ED is meant to have at least one CP, however a recent survey found this was not the case, and we don't know why. This means not everyone may get the same care, with some EDs having more senior staff than others. Across the healthcare system CPs have been good, helping to make patients better and happier when unwell.

What do we want to do?

To find out:

1. What it's like to be a CP, especially when starting as a CP, how they make a difference in UK UEC settings, as well as to find out from service-users and NHS employers.

2. What CPs think about the good and bad of starting in this role, if they feel looked after and prepared in the role, and what they would like done differently.

3. If patients like having CPs and if they think they are helpful or what would be helpful for CP roles of the future.

Together with patients and CPs, we want to create a toolkit to find out how to get CPs into all UEC services, and then look at how much good they are doing, or they can do to improve services in the future.

How are we going to do it:

There will be four parts:

1. We will look at published research on this topic
2. We will interview CPs, NHS staff who employ CPs and individuals working in integrated care boards who help put together workforce strategies across the UK
3. We will have group discussions with NHS staff in CP roles in UEC and patients/service-users who have recently accessed UEC across the UK
4. After this we will run a workshop to co-produce a toolkit to help bring in and see how good CPs are, ready to be tested in the workplace.

Public involvement and engagement:

Patients/service-users who recently accessed UEC gave their thoughts and ideas; they felt this was an important research idea because it is hard working in UEC, and there's not always enough or the right people to help patients.

A study patient and public advisory group (PAG) will be set up. Group members will be involved in developing materials for getting people involved in the study and coming up with questions for interviews and group discussions. They will help look at findings, put together workshop documents and share all study findings with the right people.

The group will meet three times in the study, and in between, researchers will share updates through email or letters.

Sharing the findings:

We will share findings in research journals, talk at local and national conferences. We will also talk about our

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findings with policymakers and professional bodies so together we can come up with the next steps, as well as test our new toolkit.

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5. Scientific Abstract

Background:

Urgent and emergency care (UEC) services are in crisis with increased demand, overcrowding and long waits placing stress onto the UEC workforce. The 2024 Darzi report identified the need for new multidisciplinary care models, with workforce growth and retention as a priority. There is evidence that poor senior clinical leadership results in worse care outcomes, quality, safety, and effectiveness of services. Senior clinical leadership can be provided by consultant practitioner roles (CP) with national workforce standards recommending at least one CP in each Emergency Department (ED). A recent survey of CP roles in the UK showed this standard was not being met, with currently only 37 ED CPs known across the UK. Consultant-level practice builds on advancing practice, offering benefits of improved patient outcomes, career progression, staff retention and stronger clinical leadership. However, reasons for the lack of CP role implementation in EDs is not known, and the impact these roles have on patients, individual EDs and the wider system has not been explored.

Aims and Objectives:

To co-produce a toolkit for the introduction, implementation and impact of consultant-level practice roles in UEC

This study will:

1. Review current literature surrounding consultant-level practice roles through qualitative systematic review with meta-synthesis

2. Explore experiences and perceptions of current CPs in UK UEC settings regarding role transition, preparedness, implementation, impact and effectiveness

3. Determine patient/service-user experiences and perceptions of CP roles in UK UEC settings, and priorities for measuring impact, effectiveness and evaluation

4. Explore macro and meso-level stakeholder perspectives of the CP role in UK UEC settings to identify implementation, impact and effectiveness metrics

Methods: The project has four inter-related work-packages (WP).

WP1: qualitative systematic review with meta-synthesis

A systematic review will be conducted to explore literature on CP role implementation, effectiveness, evaluation and impact. This will include search strategies, quality appraisal, and meta-synthesis using a synthesis matrix.

WP2: Semi-structured interviews and WP3: Focus Groups

WP2 and WP3 focus on gathering opinions and experiences of stakeholders at macro (policy), meso (organisational), and micro (individual) levels.

WP4: Implementation and impact toolkit development (informed by previous work packages)

A stakeholder workshop (12-15 participants) will co-design a implementation and impact toolkit for acceptability testing, using realist evaluation principles. The toolkit will target macro and meso groups and those pursuing future UEC CP roles, ultimately benefiting UEC service-users.

Impact:

This study will achieve impact by developing a toolkit that supports sustainable workforce planning in the NHS at consultant-practice level. We anticipate this toolkit will be transferable to other areas and will be available under an open license agreement for further validation and use.

Dissemination:

Key outcomes include four planned deliverables, with dissemination through Patient and Public Advisory Group (PAG) collaboration, conference presentations, at least three publications, and increased service-user awareness of CP roles in UEC.

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6. Detailed Research Plan

Problem being addressed:

In 2024, Health Services Research UK launched a call for action setting out the urgent need for research and evidence to support decision-making in designing the healthcare workforce. This report outlines the current challenged state of the workforce, highlighting the need for rigorous evidence to improve policy and practice in timely and actionable forms.

Urgent and emergency care (UEC) services face a crisis, with a need for evidence on managing workload, maintaining care quality, and ensuring a positive service-user experience. The workforce is the NHS' most valuable and costly resource, so evidence-driven workforce decisions are crucial for service, staff, and service-user outcomes. “The UK spends about 65% of the NHS budget on its workforce, yet it invests a tiny fraction of the public and government budget for health and care research on understanding that workforce and improving its efficiency, effectiveness, and outcomes” (Walsh et al 2024).

The Darzi report (2024) highlights gaps in the experienced UEC workforce, stressing the need for new multidisciplinary care models to meet the needs of changing UEC demand and scope. This aligns with the 2023 UEC Recovery Plan, which prioritises workforce growth and the need for strong local leadership to drive transformation. Effective use of multidisciplinary team members is crucial, including individuals from nursing and AHP clinical backgrounds, alongside developing consultant-level practice roles (NHSE 2023). To address unnecessary variation, workforce planning must take a strategic approach, aligning with local needs and be based on sustainable models. One proposed solution in addressing requirements for a senior workforce and supporting skills shortages has been the development of consultant-level practice roles. Consultant-level practice encompasses expertise across four domains necessary for enabling quality care, providing a key skill set for systems leadership and transformation through clinical credibility within a field and adoption of clinical academic roles as embedded researchers (NHSE 2023). Consultant-level practice roles now extended to nurses, paramedics, and physiotherapists, are clinical experts responsible for leadership in policy, practice, research, education, and advancing clinical practice (RCEM/RCN 2020).

There is national demand for consultant-level practice roles to meet complex health needs and improve healthcare quality, efficiency, and sustainability. Despite the policy direction for these roles there is limited evidence on their impact on patients, services or the wider health system. This has led to inconsistent implementation and more recently, challenges from other professions around their effectiveness and scope of practice. Unaddressed issues in role implementation, impact, and evaluation could lead to health inequalities, poor patient experiences, and reduced cost-effectiveness.

Rising attendances and rapidly changing care models are increasing workloads, impacting recruitment and retention in UEC, and highlighting the need to expand CP roles in this area (RCEM 2015). EDs face growing pressures from increasing patient numbers, ageing populations, and complex conditions, compounded by economic constraints and higher societal expectations (NHSE 2019, RCEM 2023). The professional Royal Colleges of Emergency Medicine and Nursing make workforce guidance recommendation of one CP per ED (RCEM 2020). UEC workforce role development and retention were recently highlighted as research priorities in a James Lind Alliance research priortisation partnership in emergency care (2022). We are focusing on the role of the CP in UEC as it is the only specialty area where a defined workforce standard for these posts exists.

Clear role definitions and theory-based evaluations are essential for successful implementation of new roles to ensure high-quality, cost-effective healthcare delivery. There is a renewed interest in consultant-level practice roles to improve patient access to emergency care but there is no existing evidence on how these roles can be successfully implemented in UEC settings, nor are there tools to determine their impact to ensure effective and equitable access for service-users.

This project provides an opportunity to develop an implementation and impact toolkit for consultant-level practice contextualised to UEC.

Why is this research important?

Whilst multiple workforce strategies have been released across the UK, evidence is needed to support and evaluate recommendations. Policymakers are hampered by the lack of research to inform strategic workforce planning particularly regarding skill and leadership required for the future workforce (HSRUK 2024). Consultant-level practice career pathways could support professional development while benefiting Trusts through the direct and indirect impact on patients and services. Without understanding the implementation, impact, and barriers to

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these roles, there is a risk of vacancies or role removal. Failure to evaluate and communicate their unique functions and impact could hinder successful deployment (Fairley 2004).

Achieving people-centered healthcare, a key WHO strategy, requires fundamental shifts in service organisation (Manley et al 2022). Historically, the Department of Health (1999) recognised consultant-level practice roles as vital for quality care, health promotion, and addressing inequalities; all critical for patient satisfaction (Crouch et al 2024). Consultant-level practice can be seen as central to transforming service delivery and better meeting population health needs (NHSE 2017). Effective care models ensure timely, appropriate care from the right providers (Horvath et al 2023). However, there is limited contemporary evidence on the implementation and impact of such roles in UEC settings specifically, and stakeholder perceptions of the role are unknown.

A gap exists between demand and recruitment in emergency care. Research priorities from the James Lind Alliance aligns with the DHSE and NIHR research focus on shaping the future workforce (2024). Optimising CP roles is essential for health service redesign and sustainability; without evidence, the role risks becoming undervalued and ineffective patient care being provided.

Review of existing evidence:

There is currently a lack of literature exploring the contemporary nature of consultant-level practice considering the development of professional roles and health services since their inception more than 25 years ago.

Policy context

Underpinning policy, the NHSE Centre for Advancing Practice released a framework for consultant-level practice, emphasising expert practice, strategic leadership, system-wide learning, and research (NHSE 2023). The framework identifies the potential for these roles to have impact but lacks guidance on role introduction, implementation, and organisational readiness.

NHS strategy highlights the need for person-centered, safe and effective services, which depend on retaining, growing, and developing the workforce. Rising demand for NHS services is straining the system, raising concerns about sustaining quality care. NHS policies focus on workforce transformation to improve service and care standards (Fothergill et al 2022), but workforce shortages affect patient experience, service capacity, and care transformation (NHSE 2023). UEC is at the forefront of recognising the value of a multi-professional workforce for high-quality care (Crouch and Brown 2018). Optimal staffing is key to timely, high-quality care and building a future-ready workforce (Drennan et al 2024) and recent NHS People Plan policies commit to utilising the full range of skills and experiences for new ways of working (Mann et al 2023).

Workforce development is key to system reform, with CPs essential for leading change and providing expert care. Clearly defined healthcare roles are needed for service-users to understand who leads, delivers, and evaluates care in collaboration with them (Crouch et al 2024). This variability is shown to hinder effective workforce planning (Evans et al 2021). Literature highlights a gap for the need to evaluate implementation and impact to solidify roles and ensure sustainable contributions to patient care (Gerrish et al 2011, Chan et al 2014), especially given variation across organisations despite shared national goals. NHS initiatives like High Quality Care for All and Equity and Excellence stress the importance of patient-centered care and understanding patient perceptions for sustainable change, goals that CPs could help achieve when embedded.

Organisational support is key for CPs to reach their full potential (Currie 2007), and literature subjectively highlights the potential benefits of CPs but there are gaps in evidence on role integration and system-wide workforce planning (Hourahane et al 2012, Kennedy et al 2012). With safe and effective emergency care key NHS and patient priority, novel solutions are needed to address workforce capacity, and growing staff pressures to ensure experienced senior clinical staff are retained (NHSE 2017). Successful implementation of national workforce policies encouraging increasing numbers of roles and from broader professions, requires attention to all levels of the health system (Taylor et al 2022). However, there is a lack of evidence regarding uptake or what is required across levels to support system-wide growth of CP roles.

Consultant Practitioner roles

UK consultant nurse roles were introduced in 1999 to support government strategic objectives on nurses' contribution to the workforce (DofH 1999, Rosser et al 2017). The role contributes to the NHS five-year forward view (2014, 2017) by supporting new career opportunities, retaining experienced staff and strengthening leadership. Additional benefits are seen as supporting innovation and strategic direction and enhancing services for patients (Crouch et al 2024). The development of the role is in response to important challenges facing healthcare systems including changing health needs of the population and rising public expectations of healthcare (Kennedy et al 2012).

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Consultant-level practice roles offer career progression by expanding practice, enhancing efficiency, and fostering collaboration. A focused review of the literature in preparing this application found research in this area broadly explores how CP roles have evolved to meet healthcare demand, and descriptions of workforce models. Literature identifies the potential of the CP role in improving care through timely access to services, mentorship for early career staff, strategic leadership, driving innovations and better patient outcomes (Kuczawski et al 2024, Dawood & Crouch 2018, Manley et al 2024). Studies suggest CPs positively impact patients' wellbeing and quality of life (Gerrish et al 2011) with greater awareness of the roles' benefits being essential for sustainable workforce planning and provision of support for effective embedment (Woodward et al 2006, RCEM 2015); however, the empirical evidence in this area is lacking.

Evidence highlights role importance in strategic influence, partnership working and embracing new practices. There is need for ongoing support from healthcare managers for continuous role development and exploration of impacts of the lack of systematic planning or policy can cause when creating a supportive workplace (Humphreys et al 2007). Organisational support is seen as crucial to maximising benefits of consultant-level practice (Woodward et al 2006), though how this is implemented in practice remains unclear. Research highlights the need to prioritise the development of consultant-level roles to support patient-centered care and align with national health priorities, such as delivering a cost-effective service that meets NHS goals (Jones 2002). A key barrier to role implementation is the failure to clearly define the unique functions and impact of these roles on health outcomes (Fairley 2004). To address this, further evaluation is needed to assess the role's impact and value within the healthcare system.

In 2004, Guest et al developed an impact framework for nurse consultants to evaluate their effect on service delivery and patient care. The framework emphasised the indirect impact on patient care through system development and staff culture. While the study found strong support for consultant-level practice, it also highlighted significant gaps about understanding role implementation. Guest's framework for consultant-level practice in England does not address context specific issues and has not been updated to reflect current working practices. The focus of the framework is solely on impact with no attention given to role implementation. It is known that inconsistent governance approaches to implementation can lead to confusion regarding roles and expectations, hindering workforce planning at scale (Taylor et al 2022). Whilst there is widespread acknowledgement that consultant-level practice roles have the capacity to impact on the experiences of both patients and frontline staff (Kennedy et al 2012), the evidence for assessing impact remains unclear.

<u>UEC-specific Consultant Practitioner roles</u>

A focused literature review on consultant-level practice roles in UEC highlighted their contribution to better patient outcomes through advanced clinical judgment and experience (Dawood & Crouch 2018). In 2017, it was suggested that CP roles could enhance healthcare access by strengthening the UEC workforce, though conditions for successful implementation, impact, and receptivity need further exploration (Woo et al 2017). A prior survey concluded diverse preparation for CP roles often leads to unclear organisational frameworks, with mismatched expectations and poor preparedness contributing to high staff turnover post-recruitment (Charters et al 2005).

While some evidence has been found regarding UEC CP roles, including to suggest the benefits of consultant-level practice roles in this setting, little is known regarding barriers and enablers to implementation, role preparedness, and stakeholder perspectives across micro, meso and macro levels. A recent grant-funded study (ConNur-ED 2024) showed that less than 1/3 of EDs had implemented such a role and that there is wide variability regarding role function but did not explore the reasons for this (Mummery & Jarman 2024).

Rising attendances and diverse care models are increasing workloads, impacting recruitment and retention in UEC, and highlighting the need to expand CP roles (RCEM 2015). There is a clear need to develop career pathways for nurses and AHPs that acknowledge clinical expertise, with innovative approaches to staffing for safer, higher-quality, and more efficient care (RCEM 2015). Research gaps have demonstrated the need to evaluate the development, alignment, and patient impact of new roles (Woodward et al 2005), with current evidence indicating that UEC CP roles face challenges in their implementation and evaluation, largely due to a lack of structured approach (Rosser et al 2017, McSherry et al 2007, Parker et al 2011). There is need for a workforce capable of leading system-level change and delivering expert care across professional domains (Crouch et al 2024), with sound understanding regarding healthcare providers' and service users' perceptions of CPs and their recognition of this practice level.

This research will draw on literature within and beyond UEC settings to address gaps and inform the proposed methodology.

<u>What will this study add to benefit service-users:</u>

Pre-submission

New perspectives will be offered on how CPs affect patient experiences and outcomes and their importance to patients/service-users in ensuring population healthcare needs are met. Identified barriers by service-users and stakeholders will inform strategies in a proposed co-produced implementation and impact toolkit for UEC settings.

This study will inform workforce policies, national agendas, and gaining professional body support. It will provide policymakers with insights into effective strategies, ensuring the successful introduction and implementation of CP roles by identifying and addressing potential barriers.

This project will build on a UK-wide RCEM funded survey undertaken by the applicant and supervisor, highlighting the lack of established CP roles and significant variability. This will improve care by providing evidence and recommendations on the implementation and impact of CP roles in UEC by:

- Understanding the contexts, mechanisms, and barriers/enablers to successful CP role implementation
- Developing a toolkit that can be used at local, regional, and national levels, to measure the patient-centered impact of consultant-practice roles
- Reduce variation by supporting equitable access to CPs in UK UEC settings, ensuring patient benefits across all four domains

Research Question

'What are understood to be the experiences and perceived barriers/enablers to role transition, implementation, and impact of multi-professional consultant-level practice within UK UEC settings to co-produce an implementation and impact toolkit?'

This research aims to address the identified evidence gap by exploring diverse perspectives on CP roles in UEC settings. It will identify barriers/enablers to role transition, preparedness, implementation and impact for current CPs and directly/indirectly involved stakeholders at micro/meso/macro level, focusing on professional expectations, motivators, KPIs, outcomes, and factors critical to successful implementation. Stakeholder consultation is key to developing and evaluating new models (Parker et al 2011). This study will capture service-users' perceptions of CP roles in UEC, exploring their impact, value, and potential to improve healthcare access.

Research will adopt an exploratory approach, emphasising qualitative methodology, to uncover perspectives and provide comprehensive descriptions of stakeholder lived experiences through questioning (Stevenson et al 2011, Mann et al 2023).

Aims and Objectives

To co-produce a toolkit for introduction, implementation and impact of CPs in UEC settings ready for acceptability testing.

This study will:

1. Review current literature surrounding consultant-level practice roles through qualitative systematic with meta-synthesis

2. Explore experiences and perceptions of current CPs in UK UEC settings of their role transition, preparedness, implementation, impact and effectiveness

3. Determine patient/service-user experiences and perceptions of CP roles in UK UEC settings, priorities for measuring impact, effectiveness, and evaluation

4. Explore macro and meso-level stakeholder perspectives of the CP role in UK UEC settings to identify implementation, impact and effectiveness metrics

Project Plan and Methods

This project was developed with the UEC national CP network and key stakeholders, including co-authors of the NHSE multi-professional consultant-level framework (2023) who provided feedback and revisions. Support has also been given by the RSS and a patient engagement group. A competitively funded RCEM grant provided preparation for this project in determining current uptake of CP roles in UK EDs.

This study comprises 4 inter-related work-packages (WP).

WP1: Systematic Review (Objective 1)

Pre-submission

A qualitative systematic review with meta-synthesis will explore literature on CP implementation, impact, effectiveness, evaluation, prevalence, and existing frameworks, led by the applicant with supervisor support as a second reviewer. Time will be dedicated to refining the review protocol, including formulating review questions using the FINER principle, designing the search strategy, and defining inclusion/exclusion criteria based on the PCC (participants, concept, context) approach (JBI online).

Quality appraisal will use CASP, with meta-synthesis managed via a synthesis matrix. PRISMA (2020) will guide the process, and Rayyan software will assist screening. These tools ensure reliability, minimise bias, and promote reproducibility (JMLA 2018). The protocol will be published as an output.

The systematic review will inform co-development of interview and focus group schedules (WP2/WP3). Whilst at this stage the final topic guide has not yet been determined, as this will be informed from WP1, through work undertaken to date potential themes may include training/transition, role support and understanding of role for example.

WP2: Interviews (Objectives 1-4)

WP2 and WP3 comprises two complimentary studies across macro (policy), meso (organisational), micro (individual practitioner) levels. It is important to capture micro, meso and macro levels to address what matters to those providing and those experiencing health services and the outcomes this brings.

Exploratory study utilising semi-structured interviews with:

1. Current UEC CPs investigating role implementation, impact, barriers/enablers, insights valuable for role transition including intrinsic/extrinsic motivational factors

2. Key stakeholders, including ICB/ICS employees, chief nurses/AHPs, directors of nursing/AHP, and UEC clinical directors from trusts with and without CP roles, to explore barriers/enablers, insights for role implementation, impact evaluation, organisational support readiness

WP3: Focus Groups (Objectives 1-4)

Exploratory study utilising focus groups with:

1. Patients/service-users

2. Consultant Practitioners

Focus groups allow in-depth exploration and understanding of actual, expected, and potential contributions of roles from multiple perspectives (Parker et al 2011). These will explore implementation barriers/enablers and impact evaluation metrics, focusing on clinician and patient/service-user perceptions linked to national frameworks. To minimise biases and ensure reliability, strategies will include a reflexive approach to critically reflect on preconceptions, relationship dynamics, and analytic focus (Cancelliere et al 2020, Galdas 2017).

Sample and recruitment

Purposive and snowball sampling will be used to select key informants for semi-structured interviews, capturing diverse perspectives and promoting rich, relevant data (Carlsen & Glenton 2011, Vasileiou et al 2018). Recruitment will be done via email through networks and advertised through NHS PPIE groups for service-users and stakeholders. Participants will be invited to contact the lead researcher for consent after receiving an information sheet. Recruitment aims for a nationally representative sample. The sampling approach will be regularly reviewed for broad representation. Interviews and focus groups will be offered in person, remotely, or hybrid.

Recruitment will target micro, meso, and macro levels, including:

• Macro: ICB/ICS regional representatives involved in UEC, professional regulatory bodies, policymakers

• Meso: Executive-level roles in NHS trusts with type-1 (EDs) and type-3 (UTCs or equivalent) departments, including chief nurses/AHPs, UEC directors of nursing/AHP/medicine

• Micro: UEC CPs, patients/service-users

Pre-submission

Sample of approximately 30 interviews is suggested to reach theoretical saturation using a semi-structured interview approach (Vasileiou et al 2018). Data collection will continue until thematic saturation is reached, where new data no longer yields new insights, and themes become repetitive. This iterative process combines sampling, data collection, and analysis until all aspects of the phenomenon are explored, supporting emerging theory (Hennink et al 2019). The study aims to recruit 30 participants across the UK for one-to-one interviews, as outlined in the SOECAT.

Literature by Hennink et al (2019) and Carlsen & Glenton (2011) suggest 2-6 focus groups are sufficient to cover 90% of themes. With 4-12 participants per group, smaller groups provide more in-depth insights. Transcripts will be reviewed after each session to refine the focus for subsequent groups. This study aims to recruit 16 participants for two focus groups, as outlined in the SOECAT.

This approach ensures a comprehensive exploration of stakeholder experiences, providing rich, triangulated data. The macro-level examines broader political, economic, and social influences, while the meso-level focuses on organisational factors shaping change. The micro-level looks at how policy, organisational, and professional factors intersect to affect CPs' implementation and impact in UEC settings (Ong et al 2014).

Data collection and analysis

In WP2 and WP3, the applicant and PAG will develop interview and focus group questions based on the systematic review. Interviews and focus groups will be recorded, transcribed, anonymised, and thematically analysed.

Thematic analysis (Braun & Clarke 2022) will identify patterns in implementation data, with phases including transcript analysis, theme development, and category confirmation (McSherry et al 2007). Impact data will be analysed using framework analysis (Gale et al 2013, Smith & Firth 2011), mapped to Guest et al's (2004) work. This existing framework (Guest et al 2004) will be used, refined and expanded where necessary to assess the impact of CP roles on patient care and outcomes, including direct/indirect effects. The SRQR checklist will guide methods and ensure research quality and transparency (EQUATOR online). Emerging themes will inform WP4.

NVivo software will be used to analyse qualitative data from both work packages to support coding and large datasets management, alongside use of a qualitative data analysis flowchart (Wong 2008). Categories/coded data will be scrutinised and discussed to ensure accuracy from transcripts. A reflexive process will be undertaken during data collection and analysis to minimise unconscious bias.

WP4: Co-Produced Toolkit

One collaborative workshop will be held. Workshop recruitment will target 12-15 participants, including CPs, stakeholders, and service-users, through UK-wide NHS PPIE groups, snowball sampling from interviews, and existing networks.

Co-design and collaboration with stakeholders will integrate key principles of improvement and participation. Matrix of framework concepts can arise from literature, interviews and focus groups. Triangulating findings from WP1, WP2, and WP3 will inform a workshop to develop an implementation and impact toolkit for UEC settings. Theoretical framework and NVivo will support data triangulation. Stakeholder feedback will assess credibility, transferability and acceptability prior to testing. Realist evaluation principles will guide toolkit development, focusing on context, mechanisms, and outcomes (Jack 2022). This approach enhances design rigor and informs policy and practice (Giles et al 2014).

Toolkit production will follow 3-steps:

1. Introduction – Assessing the needs of service-users, healthcare providers, and the health system to clarify how the role can meet these needs and ensure stakeholder alignment.
2. Implementation – Ensure policies, resources, and support are in place to introduce the role effectively, improving understanding of its impact on patients, providers, organisations, and outcomes, to promote optimal utilisation and achieve expected results.
3. Sustainability – Ensure long-term benefits and impact by identifying ongoing developments, trends, and needs for role adaptation and support to meet the healthcare system's evolving requirements.

This will:

- Include factors influencing role uptake to increase understanding on those that require attention across micro, meso and macro levels.
- Support future practice and role development by evaluating implementation and impact, offering strategies to overcome barriers, and providing a transferable evidence base for effective commissioning and delivery of CP roles in UEC settings.

Pre-submission

- Sustain the workforce, enhance quality, improve health outcomes, promote leadership, and ensure long-term cost-effectiveness of UEC CP roles.
- Enable national evaluation of CP role implementation and impact in UEC settings, identifying key conditions for success at micro, meso, and macro levels.

Impact, Outputs and Dissemination

This study aims to inform workforce planning in UEC services by collaborating with NHS colleagues and incorporating service-user perspectives. It will provide evidence to guide decision-makers on CP role implementation, career progression, retention, and improving service quality and outcomes. This study will identify enablers and barriers to implementing CP roles in UEC settings from the perspectives of CPs, service-users, employers, and workforce strategists, contributing to the literature on the need for consultant-level practice in UEC and benefiting patients/service-users.

Developing robust tools should include inter-professional and service-user engagement to ensure cost-effectiveness and quality assurance (McSherry et al 2007, NHSE 2017). An evaluative tool can support capturing the complexity, clinical significance, and impact of CP roles on service-users and professionals focusing on safety, effectiveness, patient-centeredness, timeliness, efficiency, and equity. Little is known about service-user perceptions of CPs or recognition of this practice level, which this study will add to the current literature base. Understanding this could optimise role utilisation, enhance healthcare contributions, and support employers strategically.

The NIHR impact planning template will guide year three impact/dissemination. Impact will be discussed through PAG meetings, bulletins, and supervisor updates.

Intended outcomes

- Identification of emerging and contemporary concepts to enhance Guest et al's (2004) framework
- A toolkit for introduction, implementation and impact that compliments the multi-professional consultant-level practice framework, ready for acceptability testing through future funding applications to assess transferability across UEC services including:

Identification of barriers and enablers to workforce transformation and effective implementation for UEC CP roles with potential solutions

Serving as a self-assessment toolkit for role implementation

Providing a coordinated approach to measuring and evidencing the value, benefits, and outcomes of UEC CP roles through effective implementation

- Increased recognition of the CP role and its impact on quality, safety and effectiveness
- Support for future sustainable workforce planning in the NHS at consultant-practice level. We anticipate this toolkit will be transferable to other areas and will be available under an open license agreement for further validation and use

Research findings will be shared locally and nationally with service-users, aspiring CPs, employers, ICBs/trusts, and key stakeholders, including policymakers, commissioners, and professional bodies.

Dissemination will utilise diverse methods, with PAG members, to ensure wide reach:

- Conference abstracts, posters, and presentations at local, regional, and national levels
- At least 3 peer-reviewed journal publications
- Ongoing collaboration with workshop stakeholders, collaborators, and fellowship contacts
- Raise service-user awareness of the CP role through an infographic of key study outcomes, shared with regional PPIE groups, and expanding PAG membership ahead of acceptability testing for the co-produced toolkit

Post-doctoral application

I plan to use the study's results, the co-produced toolkit, and skills gained through the training programme to support my NIHR post-doctoral fellowship application, advancing both my personal development and this field of work.

Project Management

I have successfully balanced full-time clinical with academic commitments while completing my PGCert and MRes, developing strong time management and prioritisation skills in a demanding emergency department role.

The applicant will oversee study delivery, providing regular updates to the supervisory team and seeking support as needed, using standardised reporting templates. Prior valuable experience of managing projects has been held.

Pre-submission

PAG members will advise on the project, offering perspectives and overseeing progress according to the GANTT chart milestones. Progress will be tracked monthly against milestones, in collaboration with supervisors, and in line with HEI postgraduate guidelines, including required progress reports.

Meetings have begun with the local finance manager and JRES colleagues to prepare this application and will continue to track fellowship spending and project progress, including ethics and governance, with auditable records and regular updates.

The lead applicant will manage recruitment, consent, in line with GCP. With experience in data protection from MRes and PI/APi roles, the study will follow sound research ethics, governance, and data protection policies.

Weekly timetable:

- One day/week undertaking clinical activity in collaboration with practice supervisors to achieve clinical development
- Two days/week undertaking training courses/research visits, consolidating learning gained and supervisor meetings
- Two days/week undertaking research activity/delivery

Ethics

Discussions have begun with JRES to prepare this fellowship application. Application for ethical approval will be made to local ethics groups, NHS REC and HRA through completion of IRAS for WP2 and WP3 which requires consenting processes for participants to be interviewed, participate in focus groups and workshop. The study protocol, participant information, consent form, and required documents will be completed before submission.

Success Criteria

Success measures will be integrated with milestones, tracked at each research stage. A GANTT chart will monitor timelines, with tools like the 'RDS Research Impact Guide' (2022) and NIHR impact agenda guiding outcomes, dissemination, and impact strategies in collaboration with the PAG.

Success measures/milestones

- WP1 – completion of systematic review with meta-synthesis and publication/poster/oral presentation
- Successful transfer from MPhil to PhD
- WP2/WP3 – recruitment targets met, sample representation and data/thematic saturation achieved for semi-structured interviews and focus groups
- WP4 – engagement of diverse stakeholders and development of a collaboratively agreed toolkit ready for acceptability testing
- Completion of proposed training and development programme
- Effective and successful mentorship/supervision of other individuals (academic and clinical)
- Completion of thesis write-up
- Dissemination of results to target groups, national conference presentations, and peer-reviewed journal publications
- Dissemination of results to nurse and AHP regulatory bodies
- Connections built with individuals and organisations to support long-term impact of this research, fostering collaboration, and influencing policy and practice
- Drafted application for NIHR Advanced Fellowship/Development and Skills Enhancement Award

Risks identified

- To mitigate recruitment challenges - purposive sampling will target known networks, with virtual or travel options for participants. WP3 workshops will offer hybrid participation, and translation services will be available if needed. PAG will advise on materials to support recruitment and study explanation.
- To address data saturation - data will be reviewed with supervisors, expert collaborators, and TACRI support.
- To address disagreements during toolkit production - further review with expert collaborators will occur, with adjustments made during acceptability testing post-fellowship.

Pre-submission

7. Patient & Public Involvement

Please describe how patients/service users, carers and the public have been involved in developing this proposal

Patient demand for UEC services is rising annually, while recruitment and retention of senior clinicians remains a challenge. In preparation for this application, this ongoing challenge was discussed at a local patient engagement meeting, emphasising its importance to patients/service-users framing this project proposal.

My experience in PPIE and successfully integrating this into previous projects has been valuable for this study. Patient/service-user involvement is key to understanding CP roles (NHSE 2017). During application and project plan development, members from a local patient engagement group were engaged to discuss and present initial research ideas. This engagement led to helping refine the aims and objectives. This was undertaken by mind mapping the different elements of the project to capture the intended outcomes and proposed impacts.

Local patient engagement group members included adult service-users and a carer, all with lived experience of UEC care at a London hospital, including care from a consultant nurse or consultant practitioner in the ED. Ensuring equal representation in the group is crucial. The group was socially and ethnically diverse, reflecting the local population to this NHS trust. Feedback from the group confirmed the project's relevance and potential benefit to those accessing care.

During my pre-doctoral bridging fellowship, I had the opportunity to discuss my project during the taught PPIE session. The project plan has been discussed with the RSS, through one-to-one sessions, and the plain English summary was developed with RSS PPIE members and discussed with the NHS trust's patient engagement group members. Through discussions with group members, I was able to greater understand how members would like to be involved in the proposed aspects of this fellowship proposal and came away from these discussions with the confirmation of willingness from the members to be involved at all stages of the research cycle.

Please describe the ways in which patients/service users, carers and the public will be actively involved in the proposed research, including any training and support provided

In year one, a Patient and Public Advisory Group (PAG) will be formed for the fellowship, with the goal of continuing involvement during acceptability testing of the proposed toolkit. Patient engagement group members involved in the proposal will be asked about their willingness to continue participating.

Opportunities with RDNs will be explored to enhance PPIE and recruit future members. If needed, PPIE opportunities will be shared via NIHR's 'People in Research' platform to reach wider representation. PAG involvement will be sought UK-wide to ensure diverse representation of UEC service access and experiences, with the NIHR RSS EDI toolkit supporting equal participation. T This project aligns with NIHR EDI definitions and the INCLUDE framework to ensure inclusive representation in the PAG, study population, and demographic reporting during data analysis. Equal representation is essential due to the diverse UEC service-user population. UK public involvement standards and research inclusion principles will be followed throughout the study (NIHR 2023).

The PAG will meet formally three times a year for task-based meetings, with email/paper updates in between. Meetings will be scheduled for broad participation, with roles and responsibilities collaboratively defined.

Membership and meetings include engagement and involvement with:

- Developing recruitment and service-user facing materials, and scheduling interviews/focus groups based on themes identified in the systematic review
- Data analysis to ensure accurate theme representation from interviews/focus groups, contributing to the development/enhancement of conceptual framework
- Pre-workshop meetings will support co-planning, ensuring inclusive participation and fostering creativity
- A leading role in disseminating study findings and driving knowledge mobilisation for UEC workforce transformation
- Reviewing drafts for publication submission

PAG members will be offered educational support. This will include opportunity for introductory teaching to be undertaken by myself for PAG members to transfer knowledge I gain from my bespoke training and development programme to others, alongside signposting or providing access to online short courses relative to the study to meet individual learning needs or areas of interest. Funding for participation has been included within the detailed budget.

Pre-submission

If it is considered not appropriate and meaningful to actively involve patients/service users, carers and the public in your proposed research, please justify why
N/A

Pre-submission

8. Training & Development and Research Support

Proposed training and development programme

The following bespoke programme, aligned with the RDF, complements the research study and addresses gaps while enhancing strengths developed through previous education and training. This will prepare me within the fellowship and beyond, towards becoming an expert clinical academic in my field. Extended justification is provided in the detailed budget.

Aims:

1. To gain skills in research design, delivery, and analysis at doctoral and post-doctoral levels
2. To develop skills in conducting systematic reviews, meta-synthesis, and qualitative research
3. To develop as a clinical-academic in my field, aligning with RDF domains and key attributes like motivation, resilience, innovation, and collaboration
4. To further develop my advancing practice career, including transferable leadership skills between academia and clinical
5. To build confidence and competence in teaching, mentorship, and co-supervising
6. To advance skills of others in UEC roles within the research pillar

City St George's Health Services Research PhD/MPhil Enrolment

City St George's is uniquely positioned alongside a leading hospital, making it an ideal choice given its proximity to my employer and existing links I wish to continue developing. Close collaboration between clinical and academia offers opportunities that will support me during the fellowship and beyond, with access to both taught and self-directed resources including methodologies and key concepts relevant to health services research. The programme sits within a department which focuses on health policy and on equipping next generations of researchers to inform healthcare policy and influence practice and organisation of care providers, thus complementing my research proposal.

Year One:

City St George's Graduate skills training programme for PhD students (one day/week 14weeks)

A comprehensive programme offering weekly sessions on research methods, project planning, and scientific writing to enhance my skills in each area.

Systematic Review

University of Bristol Medical School:

- Introduction to systematic reviews and meta-analysis (4days)- Led by experts in systematic review, this will enhance my ability to conduct searches, use tools, summarise findings, and evaluate quality.

Qualitative Methods

University of Oxford Department of Primary Care Health Sciences:

- Introduction to qualitative research methods (5days), doing qualitative interviews (1day) and analysing qualitative data (2days) - Delivered by qualitative experts, this will equip me with the skills to apply study methods and provide transferable knowledge for the future.

Social Research Association:

- Introduction to focus groups, conducting focus groups and conducting online focus groups (1day/course) – Learn skills to design and run groups, including moderation and managing group dynamics

PPIE

ARC South London:

- King's Improvement Science introduces: the value of partnering with the public to improve health services (1day)

Imperial College London

- Public Involvement Training (online)

To build on PPIE skills, ensuring they are embedded throughout the research cycle, and support the development of the PAG.

Year Two:

Reflexivity

Pre-submission

Social Research Association:

- Reflexivity in qualitative research (2days) - No previous training in reflexivity, therefore this will provide essential skills and experience for qualitative research including minimising biases

Qualitative Data Analysis

City St George's, University of London:

- Introductory and Intermediary training courses in NVivo for qualitative and mixed-methods analysis - No prior training in qualitative data analysis software; this offers practical experience in using the tool. Accessed through enrolment as PhD student at City St George's

Advancing Practice - Leadership

- King's Fund Emerging Clinical Leaders programme (6days over 4months) - Develop leadership skills in compassionate team cultures, patient-centered care, team wellbeing, and personal resilience for future more senior roles

Year Three:

Workshop – co-production

Point of care foundation:

- Experience-based co-design (1day) - Structured learning will build on skills gained through my CLAHRC role, enhancing my ability to incorporate others' perspectives in designing change—critical for the stakeholder workshop and toolkit design.

Dissemination and Impact

- NIHR 'introduction to knowledge mobilisation', 'engaging with policy makers' and 'mobilising research knowledge with members of the public' webinars (online)
- The Health Foundation 'communicating your research' (online)

Recommended by the RSS, these courses will equip me with the knowledge, skills, and innovative methods to ensure impactful and widespread dissemination of study outcomes.

Writing

- ARC South London PhD Writing Course (1day) - Scientific writing, particularly writing succinctly, is a weakness I aim to address. This was a challenge during my MRes, and I need further experience in writing to word limits at PhD level

NIHR advanced fellowship/development and skills enhancement award webinar/application day – An opportunity to understand post-doctoral fellowship requirements, advance study outcomes, build collaborations with the NIHR Academy, and progress my clinical-academic career.

Research Visits:

- Visit to NHSE/Centre for Advancing Practice to deepen understanding of health policy development, impact, and workforce
- Visits to national ICBs to enhance understanding of healthcare strategy and develop strategic thinking skills
- Visits to ARCs, RDNs and Health Services Research UK to deepen understanding of research strategy/agenda across the UK
- To remain active in the ED CRG, developing mentorship and supervision skills for clinical research nurses/AHPs within the department and trust, in collaboration with the NMAHP research lead and TACRI

I am requesting financial support to attend conferences for networking, learning about advances in care, discussing my research, and supporting dissemination. Justification and costs are detailed in the budget.

- RCEM Annual Scientific Conference
- RCN Research Conference
- CSP Conference

I will access free online courses and webinars, including those from NIHR and RSS, and attend CAHPR regional/national and APPN meetings with supervisor support. These events offer networking and potential future executive committee opportunities.

Peer to peer support - I will attend and present work at the regular seminar series hosted by the Centre for Healthcare Innovation Research, City St George's.

20% clinical practice:

Opportunity to work clinically with consultant-level practitioners and practice supervisors to map my skills against consultant-level practice roles and develop the expertise needed as a future UEC CP. To seek coaching and observation opportunities with current role holders, focusing on strategic thinking, while enhancing my advancing

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practice skills and sharing knowledge within my department and seeking clinical secondment opportunities in other UEC services. This includes completing the NHSE e-portfolio to evidence advancing practice, working towards the digital badge. I will also collaborate with trust-level ACP leads and continue my role as joint trust independent prescribing lead.

Primary Doctoral Supervisor

PRIMARY DOCTORAL SUPERVISOR 1

Name	Dr Raheelah Ahmad

Institution	City St George's, University of London

Position	Associate Professor

Description of support provided
Dr Raheelah Ahmad is a Reader/Associate Professor in Health Systems in the Department of Health Services Research & Management, School of Health & Medical Sciences at City St George's University of London (CSG UoL). Raheelah has over a decade of mixed method research and evaluation of health systems and change in the UK and international settings and heads up the workforce innovation lab at the Centre for Healthcare Innovation Research (CSG UoL). Raheelah has been recipient of an NIHR Knowledge Mobilisation Fellowship award and is Co-I on three NIHR Health Protection Research Units at Imperial College London (through 2 commissions). She is Co-I on the third commission of the HPRU Respiratory Infections continuing from March 2025. Raheelah's technical advisory roles include the Technical Advisory Group UK DHSC/GOV Fleming Fund, Deputy Chair Wellcome Trust Surveillance and Epidemiology of Drug-Resistant Infections. Raheelah has been principal supervisor for 6 PhD students to successful completion, and secondary supervisor for 3 students and currently primary for 2 students. She has supervised over 50 MSc Dissertation students in Public Health and Health Management and the MSc MRes and is formal mentor to Early Career Researchers within the department. Raheelah has worked with the Professor Jarman to review the evolving application and to identify complementary roles and expertise from across the supporting team. They will continue to co-supervise through monthly joint meetings and drawing in opportunities for critical feedback and peer support from collective students and trainees.

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Additional Academic Supervisor(s)

ADDITIONAL ACADEMIC SUPERVISOR 1

Name	Professor Heather Jarman

Institution	St George's University Hospital NHS Foundation Trust

Position	Group Director for Health and Care Professions Research, Professor of Emergency Care

Description of support provided

Heather Jarman is Director for Health and Care Professions Research at St George's Trust and Professor of Emergency Care at City St George's, University of London. Her full-time substantive NHS role is balanced between strategic leadership and as a clinical academic in emergency care (EC).

Professor Jarman has a research interest in workforce having explored nursing roles in major trauma for her doctorate. She is co-applicant/expert panel chair on an NIHR Health and Social Care Delivery Research grant exploring EC advancing-practice roles.

She is currently supervising a NIHR Doctoral Research Fellow, two NIHR Academic Clinical Fellows, and an NIHR PCAF. She has supervised 4 MSc students to completion and advises on grant applications and fellowship awards for numerous AHPs in the Trust. She has examined 3 qualitative/mixed methods PhDs. Professor Jarman is mentoring 3 NIHR DCAFs and 1 PCAF.

Professor Jarman acted as my pre-doctoral programme supervisor and her influence has shaped this application. We have collaborated on the preparation of 3 accepted conferences abstracts and 2 papers (both in review). She is lead/supervisor on the grant-funded project on consultant-practice roles in ED that is the precursor to the work planned in this DCAF.

With Dr Ahmad, we have demonstrated an effective supervision relationship in preparing this application, with regular meetings and email communication. They will continue to co-supervise through monthly joint meetings with Professor Jarman providing subject and methodological expertise. Meetings will be supplemented by written feedback on submitted drafts.

ADDITIONAL ACADEMIC SUPERVISOR 2

Name	Professor Robert Crouch

Institution	University Hospital Southampton/University of Southampton

Position	Consultant Nurse and Honorary Professor

Description of support provided
Robert Crouch is a Consultant Nurse & Honorary Professor of Emergency Care, University Hospital Southampton/University of Southampton, School of Health Sciences. Visiting Professor, Faculty of Health and Applied Sciences, University of the West of England. He has contributed significantly to the development of advanced roles, in particular, the Royal College of Emergency Medicine Advanced Clinical Practitioner programme. He has an interest in pre-hospital care; he previously worked as Consultant Practitioner with the Hampshire and Isle of Wight Air Ambulance service HEMS team; he now contributes as a Community First Responder for Southwest Ambulance Service. Working with NHS England, he has contributed to the national development framework for advanced practice and is the NHS England Senior Clinical Lead for Multi-professional consultant-level practice. In his research leadership role, he leads the research programme at Dorset and Somerset Air Ambulance. He is asked to regularly review research grant applications for the NIHR and other grant awarding bodies and was a panel member for the NIHR RfPB Under-represented disciplines Nurses and Midwives. He is currently mentoring a NIHR DCAF applicant and has considerable experience in Doctoral supervision and examinations. Professor Crouch and I have met to review the project proposal, with Professor Crouch providing feedback and revisions. Professor Crouch will join scheduled supervision sessions with Dr Ahmad and Professor Jarman providing subject expertise. Professor Crouch will support with proposed research visits to NHSE Centre for Advancing Practice through his role as clinical lead for consultant-level practice.

Clinical Supervisor(s)

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PRACTICE SUPERVISOR 1

Name	Dr Tim Noblet

Institution	St George's University Hospital NHS Foundation Trust

Position	Consultant Physiotherapist, Adjunct Professor of Research, Honorary Associate Professor

Description of support provided
Dr Noblet will provide strong mentorship and practice support to develop my physiotherapy profession needs and support my continued development as a clinical-academic. His mentorship is key in supporting my development within all pillars of advancing practice and in mapping my areas of progression towards consultant-level practice in the future. Dr Noblet works in a consultant-level practice and clinical academic role at St George's University Hospitals NHS Foundation Trust and is therefore well placed to support my development. He is committed to developing clinical academics and supporting advancing practice physiotherapists. Dr Noblet has experience working with Health Education England and he is currently working with an international collaboration to develop international expert practitioner, and clinical academic frameworks for physiotherapy which highly compliments my research study. Within his clinical academic role Dr Noblet supervises and mentors Professional Doctorate and PhD students across the UK and internationally. His involvement in numerous collaborations will support my development of strategic thinking skills and my wish to undertake executive membership of advancing practice groups in the future. I will meet with Dr Noblet monthly, initially undertaking a formal training needs analysis for the duration of this three-year fellowship and then to work collaboratively on my clinical academic development through local projects, networking and supporting access to opportunities for wider experiences regionally and nationally.

PRACTICE SUPERVISOR 2

Name	Dr Edward Baker

Institution	King's College Hospital

Position	Consultant Nurse, Honorary Senior Lecturer

Description of support provided
Dr Baker is an experienced consultant nurse working at King's College Hospital ED and holds an honorary Senior lectureship in the Centre for Trauma Sciences at Queen Mary University of London. He completed his PhD supported by a NIHR/HEE CDRF in 2021. Since completing his PhD, Dr Baker participated in a HEE post-doctoral bridging scheme and is in the process of applying for a NIHR SCPRA. Dr Baker plans to undertake the NHSE e-portfolio scheme in due course, which coincides with my application and therefore providing peer support through this process. Prior to taking up his research fellowship, Dr Baker was a lecturer practitioner teaching on the Advanced Practice programme at King's College London. Dr Baker was voted onto the NIHR Emergency Care Incubator steering committee, is a RCEM Research Committee member, contributes to several clinical working groups for the London Major Trauma System and is an associate editor for International Emergency Nursing Journal. Having collaborated with Dr Baker as a co-applicant on the ConNur-ED study, we have already established a working relationship. Dr Baker will support my development as a clinical-academic specifically within UEC and through the continuum from advancing to consultant-level practice within this setting.

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I will meet with Dr Baker monthly, initially undertaking a formal training needs analysis for the duration of this fellowship and then to work collaboratively on my clinical-academic development through skills enhancement in line with the four practice pillars, networking and supporting access to opportunities for wider experiences regionally and nationally.

Collaborations

Emergency Nurse/Practitioner Consultant Group UK

This professional network links together the consultant nurses and consultant practitioners working in urgent and emergency care settings to share best practice and provide expertise consultancy on emergency care in the UK. Four members of this group have provided feedback on the project proposal and have agreed to continue to support the project / act as stakeholders if successful. The group will be key to the success of this work both as key participants and informants, but also in peer review, feedback and dissemination.

Professor Kim Manley

Professor Manley's expertise is in practice based interdisciplinary collaborative research that integrates research, practice and learning. She has a particular focus on leadership development, learning and improvement, system and workforce transformation to develop and sustain workplace cultures that are person centered, safe, effective and good places to work. Her expertise aligns with my topic and research subject interests. She was a co-producer of the centre for advancing practice consultant-level framework. Professor Manley has supported the development of this research proposal and agreed to provide continued support and collaboration throughout the fellowship, including supporting through links to the Centre for Advancing Practice consultant-level practice group and as an expert in research in this field.

Professor Guest and co-authors of 'Guest et al's Consultant Nurse impact framework (2004)':

Professor Guest is Professor at King's Business School. His research and writing address the relationship between human resource management, organisational performance and employee well-being, the changing role of careers and quality of working life. Communication has been held on utilisation of Professor Guest et al's published impact framework (2004) that aims to be built upon as an outcome of this study and being a point of contact for expert advice as required.

Translational and Clinical Research Institute (TACRI)

A collaboration between St George's NHS Trust and City St George's University, TACRI supports the development of research collaborations, interests, skills and career progression. TACRI will be a point of contact and support by providing educational and clinical seminars and access to an expert consultancy service, where I will be able to sense-check methodologies and data analysis approaches. Additionally, as an existing member I would like to gain experience in greater involvement in the institute for my future clinical-academic career developing the research leadership skills required.

Dr Mary Halter

Dr Mary Halter is an applied health services researcher, working in Kingston University's Emergency Cardiovascular and Critical Care Research Group. She has collaborated on and led research associated with the NHS, its workforce and service users including SkillMix-ED; a NIHR funded study which I am an expert panel member for. Having built a working relationship with Dr Halter through prior work, her expertise links closely with my research topic and interests for my future clinical-academic career. Dr Halter will support extending my health services research experience and knowledge and, translating my learning gained from SkillMix-ED study design into my own research.

Advanced Physiotherapy Practitioner Network (APPN)

APPN supports advanced practice roles across all sectors and specialties of physiotherapy practice in the UK. In conjunction with my practice supervisor, Dr Noblet, who is an executive member of this network APPN will provide peer support during both my clinical and academic training plan with opportunity to network and collaborate with

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others in extended scope physiotherapy roles across the UK for my professional development. APPN has links with national bodies supporting planned visits within my training and development plan for me to gain wider understanding on UK regulation and impact/influence on a strategic level.

Host Organisation Support Statement
The following statement of support should be composed jointly by the Heads of Department / Senior Managers of the employing host organisation and the partner organisation.
The Heads of Department / Senior Managers of these organisations will be able to add the statement of support to the form once they have confirmed participation. Invitations to participate are sent by the applicant via the 'Participants and Signatories' section of the form.

Statement of support

We confirm as senior managers at St. George's University Hospital NHS Foundation Trust (SGUHFT) and City St George's, University of London, that we are fully supportive of Victoria's proposed research and application for an NIHR Doctoral Clinical Academic Fellowship.

As NHS host institution and clinical partner, SGUHFT is one of London's biggest hospital Trusts, providing services to 3.5 million people with a staff of over 9000. The Trust is committed to promoting equality, diversity and inclusion which is represented in the Trust values and through the NHS workforce disability equality standards. The senior leadership team in the Trust recognise the critical role that the allied health professions have in expanding research capacity, patient involvement, patient outcomes and experience. The Trust has developed a sustainable programme of training and research to increase this research capacity, literacy and capability; to this end the Trust research strategy 2023-2028 makes specific reference to investing in staff wishing to pursue clinical academic careers by funding training on research skills and methods, and in providing protected time for research.

The Translational and Clinical Research Institute (TACRI) hosted by St. George's University Hospitals NHS Foundation Trust with City St George's supports researchers at all levels engage in research and grow our academic and clinical partnerships. We have an established track record of supporting clinicians who wish to become research active. Regular research seminars showcase research by Trust staff and research training is provided through bespoke workshops which would support Vicky in her Fellowship e.g. patient and public involvement in research, funding opportunities. The Trust has a doctoral and post-doctoral research forum of which Vicky will be part and which exposes her to a wider network of allied health professionals who are also who are also developing and succeeding in clinical academic careers. Under this strategy the number of successful health and care professions research fellowships has grown with current NIHR PCAFs (x5), DCAFs (x4), Senior Practitioner Award (x1) and ACAF (x1) in the Trust.

Under the guidance of Dr Ahmad, this DCAF will be supported by the Centre for Healthcare Innovation Research (CHIR) within the School of Health & Medical Sciences (SHMS) at City St George's, a multi-disciplinary school with research institutes in Population Health Research, Infection & Immunity, Cardiovascular Sciences & Genomics, Neurosciences & Cell Biology, and research centres in Healthcare Innovation, Mental Health, Maternal & Child Health, Language & Communication Sciences, Applied Visual Sciences, and Food Policy, and Clinical, Social & Cognitive Neuroscience. In the 2021 REF assessment period, in the Allied Health Professions & Nursing unit of assessment (UoA3), our researchers secured funding from 89 funding bodies (e.g., NIHR, UKRI, Dunhill Medical Trust, Wellcome Trust) totalling £15.6 million and produced more than 2100 research outputs. 91% of our UoA3 research outputs were rated as world-leading (4*) or internationally excellent (3*), 100% of our submission rated as outstanding (4*) or very considerable (3*) for impact, and 100% of our research environment ranked 4* or 3*.

CHIR is well placed to support the underpinning research and co-design of a toolkit supporting the implementation and impact of workforce roles, hosting considerable expertise in behaviour change and complex intervention development, study design and workforce re-design, complemented by experts in statistics in the Population Health Research Institute. Victoria will have access to regular research centre meetings where centre members present research ideas in development, critically discuss and learn from each other. Doctoral researchers are an important part of this group and have regular opportunities to present and receive feedback on their research. Activities provided by the centre include training and career development workshops, facilitated writing days, student presentation days, and access to methodological experts. Victoria will also have access to a regular series of Early Career Research development workshops, and a dedicated area for research students.

City St George's is fully committed to Equality, Diversity and Inclusion employment practices, as emphasized in our 2030 Strategic Vision and overseen by the Dean for Equality, Diversity and Inclusion. We are committed to the VITAE Concordat to support the career development of researchers and to the principles of the European HR Excellence In Research Award (HREIRA). City St George's has held an Athena Swan Silver Award since 2018

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and is a Stonewall Diversity Champion and Disability Confident Employer.

This application aligns with the university's 2030 Strategic Vision, which includes conducting research at the frontiers of practice; influencing the health policy agenda in the UK and globally; increasing the involvement of the public and patients and use of co-design in our research activities; supporting our academics to develop their research profile, through recognition by external bodies and stakeholder groups; and focusing on delivering research with high quality impact. City St George's has both ARC South London and ARC North Thames membership which further facilitates co-production of research through patient and public involvement in research.

Victoria has demonstrated a long-standing commitment to academic practice and was awarded a competitive Trust-funded Research Fellowship in 2023 in recognition of the strength of her potential as a future academic leader. She used the time awarded on the Fellowship, in addition to the NHSE NIHR pre-doctoral bridging scheme, to develop her DCAF application and expand her research skills and knowledge through conference presentations and being a Principal Investigator on a NIHR-funded study.

The Trust has an agreed pathway for the development of clinical academic posts at pre- and post-doctoral level, further supported by the George's Academic Training (GAT) programme, and in the medium term we aim to support Victoria beyond a successful DCAF and PhD by developing a specific clinical academic role for her to apply for on completion of this fellowship. The emergency department collaborative research group team in which Victoria is based has a strong multi-professional academic track record with two NIHR academic clinical fellows working alongside a Professor of Emergency Care. She will continue in her practice role for 20% of her time on the fellowship and remain fully integrated within the clinical team.

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9. Detailed Budget

The finance section should provide a breakdown of costs associated with undertaking the research as described in the proposal.

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Staff type	Lead Applicant

Name	Victoria Mummery

Employing organisation	St George's University Hospitals NHS Foundation Trust

Grade	AFC Band 8A

Starting point on scale	4

Increment Date	08/11/2025

Justification of Costs
This detailed budget has been completed collaboratively with JRES finance advisor Adam Bales, research governance team and my supervisory team. In addition, the SOECAT was completed in conjunction with the JRES governance manager. Salary costs provided by the St George's Joint Research and Enterprise Service based on the lead applicant's current salary within the trust. AFC Band 8A Point 4 at 100% FTE for 36 months, incrementing on the 8th November. The budget considers the research proposal and training and development programme that meets the project's, PPIE and applicant's learning needs to effectively and successfully undertake the study, fellowship and prepare for future for my future clinical-academic career. The training and development programme consists of highly recommended courses and workshops, research visits and conferences, alongside PhD course tuition fees. These are essential to produce and disseminate high quality and impactful research, alongside developing the required skills to be a doctoral researcher. The budget includes essential PPIE activities for the duration of the fellowship and has been costed appropriately through use of NIHR INVOLVE guidance. The costs included for this fellowship will provide assurance that the proposed outputs, both research and personal development, detailed within this application are successfully achieved and most importantly, the proposed benefits are provided to our service-users to improve the UEC workforce and UEC services across the UK.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	56454
Geographical Weighting (£)	8172
Other allowances (£)	

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	Total (£)
Superannuation (£)	9293.22
Nat. Insurance (£)	8070.50
Current Annual Costs (£)	**81989.72**

Direct Costs
These are the costs that are specific to the research, which will be charged as the amount actually spent and can be supported by an audit record.

Staff Type: (Lead Applicant)-Miss Victoria Mummery
Type of cost: NHS
% Full-time on this research: 100.0
Total months on this research: 36.0
Justification of Cost: Salary costs provided by the St George's Joint Research and Enterprise Service based on the lead applicants current salary within the trust. AFC Band 8A Point 4 at 100% FTE for 36 months, incrementing on the 8th Nov.

	Year 1	Year 2	Year 3	Total Cost (£)
Basic Salary	56454	59492	60504	176450
National Insurance	8071	8509	8655	25235
Superannuation	9293	9730	9875	28898
Geographical weighting / Other allowances (if applicable)	8172	8172	8172	24516

	Year 1	Year 2	Year 3	Total Cost (£)
Total HEI	0	0	0	0
Total NHS	81990	85903	87206	255099
Total Commercial	0	0	0	0
Total Other	0	0	0	0

Travel, Subsistence and Conference Fees

Type of cost: NHS
Description: Conference Fees
Conference: True
Item Description: Royal College of Emergency Medicine Annual Scientific Conference
Justification of Cost: Nationally recognised conference relevant to my clinical expertise and area of clinical academic field. I have had opportunity to attend this conference once before and found this significantly beneficial to my learning, ability to network and to mobilise new knowledge into practice. To attend in year three to open opportunities for networking within clinical field and learn from others currently undertaking local and national research projects on clinically relevant topics and provide opportunities for discussion of research study and early results dissemination. I plan to submit for poster presentation/oral presentation at this conference.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	600	600

Type of cost: NHS
Description: Journey Costs
Conference: True

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Item Description: Travel to Royal College of Emergency Medicine Annual Scientific Conference
Justification of Cost: To support travelling to RCEM ASC in year three for networking opportunities, research project discussion with others in same clinical field, updates for clinical practice and early dissemination of results. Attendance at this conference is with those in the same clinical field as myself. The conference is held within the UK and therefore travel has been costed from London to location.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	150	150

Type of cost: NHS
Description: Subsistence
Conference: True
Item Description: Accommodation for Royal College of Emergency Medicine Annual Scientific Conference
Justification of Cost: RCEM ASC is usually held over three consecutive days and therefore the below supports accommodation costs for attendance to the conference in year three of the fellowship

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	200	200

Type of cost: NHS
Description: Conference Fees
Conference: True
Item Description: RCN International Nursing Research Conference
Justification of Cost: Opportunity to attend research specific conference to improve practice, learn, network and discuss current challenges within healthcare. To be able to discuss my research study with others and seek networking opportunities with others in my clinical and research field

	Year 1	Year 2	Year 3	Total Cost (£)
	582	0	0	582

Type of cost: NHS
Description: Journey Costs
Conference: True
Item Description: Travel - RCN International Nursing Research Conference
Justification of Cost: To support travel to this conference which will provide me with the opportunity to attend a research specific conference, network with others, update my practice and discuss my research study. The conference is located within the UK and therefore travel has been costed from London to UK location.

	Year 1	Year 2	Year 3	Total Cost (£)
	150	0	0	150

Type of cost: NHS
Description: Subsistence
Conference: True
Item Description: Accommodation - RCN International Nursing Research Conference
Justification of Cost: To support accommodation to be able to attend this three day conference which will provide me with the opportunity to attend a research specific conference, network with others, update my practice and discuss my research study

	Year 1	Year 2	Year 3	Total Cost (£)
	200	0	0	200

Type of cost: NHS
Description: Conference Fees
Conference: True
Item Description: The Chartered Society of Physiotherapy Annual Conference
Justification of Cost: To have opportunity to attend profession specific annual conference to network and collaborate with other physiotherapists working in advancing practice and consultant-level roles and for early discussion/dissemination of research study findings. I plan to submit for poster/oral presentation for this conference.

	Year 1	Year 2	Year 3	Total Cost (£)

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	0	0	179	179

Type of cost: NHS
Description: Journey Costs
Conference: True
Item Description: Travel - The Chartered Society of Physiotherapy Annual Conference
Justification of Cost: To support with travel to the annual CSP conference which is a conference specific to my profession for opportunity to network, learn and develop my practice, discuss preliminary research study findings and opportunity for collaboration for others on next steps of the study. The conference is located within the UK and therefore travel has been costed from London to UK location.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	150	150

Type of cost: NHS
Description: Subsistence
Conference: True
Item Description: Accommodation - The Chartered Society of Physiotherapy Annual Conference
Justification of Cost: To support with accommodation to be able to attend the two day annual CSP conference which is a conference specific to my profession for opportunity to network, learn and develop my practice, discuss preliminary research study findings and opportunity for collaboration for others on next steps of the study

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	200	200

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Travel Costs - Research Visits
Justification of Cost: To support for travel to proposed research visits including to NHSE and the Centre for Advancing Practice, Integrated Care Boards regionally/nationally and NIHR regional research delivery networks to increase my understanding of health policy development, workforce strategy, research strategy and strategic thinking skills. Locations will be across the UK and therefore cost reflects travel from London to selected regions.

	Year 1	Year 2	Year 3	Total Cost (£)
	250	250	250	750

Type of cost: NHS
Description: Subsistence
Conference: False
Item Description: Accommodation Costs - Research Visits
Justification of Cost: To support accommodation during proposed research visits including to NHSE and the Centre for Advancing Practice, Integrated Care Boards regionally/nationally and NIHR regional research delivery networks to increase my understanding of health policy development, workforce strategy, research strategy and strategic thinking skills. Locations will be across the UK and therefore cost reflects accommodation with selected regions.

	Year 1	Year 2	Year 3	Total Cost (£)
	250	250	250	750

Type of cost: NHS
Description: Journey Costs
Conference: False
Item Description: Point of Care Foundation - Experience-based co-design course - Travel
Justification of Cost: To support with travel costs to attend the one-day in person experience-based co-design course to provide me with the essential skills to run the proposed stakeholder workshop within the study and to provide skills for any future experience-based co-design research/teaching opportunities. Location of this course is within Central London, therefore easily accessible with the reduced travel costs reflecting travel from living address to central London.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	50	50

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	Year 1	Year 2	Year 3	Total Cost (£)
Travel, Subsistence and Conference Fees Total	1432	500	2029	3961

Equipment

Type of cost: NHS
VAT relief to be applied: False
Description: Digital Audio Recorder
Price excluding VAT £: 100
Expected life time: 36
Justification of Cost: To be able to effectively record interviews, focus groups and workshop to ensure all data is gained accurately ahead of transcribing and inputting into NVivo. Recordings will be required to be sent for transcription. The prices is based on a recommended middle of the range known brand.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	100	0	0	100

Type of cost: NHS
VAT relief to be applied: False
Description: Laptop
Price excluding VAT £: 1000
Expected life time: 36
Justification of Cost: To be able to undertake digital requirements and electronic documents of the fellowship including the PhD programme, supervision sessions, short courses, conferences, data analysis, presentations. This will support HEI ability to download NVivo to this secure device. All appropriate documents will be able to saved into one secure device for accessibility. Will support dissemination and next steps after fellowship completion. The laptop is required to have appropriate encryption capabilities, high memory capacity and processing speed, and the ability for remote NHS/hospital account access for entirety of fellowship. The price is based on a recommended middle of the range known brand.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	1000	0	0	1000

Type of cost: NHS
VAT relief to be applied: False
Description: Memory Sticks
Price excluding VAT £: 20
Expected life time: 36
Justification of Cost: 2 x memory sticks for encrypted data storage in line with governance processes/policies. Price based on recommended middle of the range known brand and will be used for the entirety of the fellowship.

	Year 1 (exc VAT)	Year 2 (exc VAT)	Year 3 (exc VAT)	Total Cost (£) (exc VAT)
	20	0	0	20

	Year 1	Year 2	Year 3	Total Cost (£)
Equipment Total	1120	0	0	1120

Consumables

Description: Printing/Photocopying of Documents
Type of cost: NHS
Please describe fully: Printing/Photocopying of Documents
Justification of Cost: Printing costs of recruitment posters, consent forms, study/participant information leaflets, resources for stakeholder workshop, study bulletin updates for public and patient advisory group where required for accessibility and in differing languages if needed. To support ability to reach widespread representative

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participation by providing documents, specifically recruitment posters, across UK NHS services. Where possible printing will be kept to a minimum to avoid unnecessary costs/use of resources.

	Year 1	Year 2	Year 3	Total Cost (£)
	50	50	50	150

Description: Voucher per service-user participant
Type of cost: NHS
Please describe fully: Voucher per service-user participant
Justification of Cost: To be able to provide a £25 voucher for participation from service-users in: Focus groups - 12 service-user participants Stakeholder workshop - 6 service-user participants

	Year 1	Year 2	Year 3	Total Cost (£)
	0	300	150	450

Description: Poster Printing for Results Dissemination
Type of cost: NHS
Please describe fully: Poster Printing for Results Dissemination
Justification of Cost: Costings for 3 x posters to be printed (systematic review, interview/focus group results, evaluative tool) at approximately £40 per poster for submission to conferences costed to attend.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	40	80	120

Description: Dissemination of information/results to PAG members and key stakeholders
Type of cost: NHS
Please describe fully: Dissemination of information/results to PAG members and key stakeholders
Justification of Cost: To cover costs of dissemination materials for both PAG members and key stakeholders of study information and results towards the end of the fellowship as detailed in the dissemination, outputs and impact plan. These are separate materials to the planned research posters for submission to conferences. These will be both plain English and scientific updates to ensure readability and will be shared widely.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	150	150

	Year 1	Year 2	Year 3	Total Cost (£)
Consumables Total	50	390	430	870

Patient and Public Involvement

Description: Public and Patient Advisory Group - Meeting Attendance
Type of cost: NHS
Please describe fully: Public and Patient Advisory Group - Meeting Attendance
Justification of Cost: 3 x task-based one hour meetings to be held over the 3year fellowship period with the public and patient advisory group (online or face to face or hybrid). Involvement of 6 members planned, with provision of £100 per meeting per member to include time, travel expenses and carers costs. Costings have been reviewed in conjunction with NHIR INVOLVE guidance

	Year 1	Year 2	Year 3	Total Cost (£)
	600	600	600	1800

Description: Public and Patient Advisory Group - Voucher
Type of cost: NHS
Please describe fully: Public and Patient Advisory Group - Voucher
Justification of Cost: To be able to provide each public and patient advisory group member (6 members) with a voucher (£25) following attendance at each of the 3 x planned meetings across the study period for their on-going collaboration with the study. Costings have been reviewed in conjunction with NHIR INVOLVE guidance.

	Year 1	Year 2	Year 3	Total Cost (£)
	150	150	150	450

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Description: Public and Patient Advisory Group - Training
Type of cost: NHS
Please describe fully: Public and Patient Advisory Group - Training
Justification of Cost: To be able to provide each public and patient advisory group member (6 members) with an £200 allowance to have the opportunity to attend a training course relevant to the their learning/interest needs and the study. Costings have been reviewed in conjunction with NHIR INVOLVE guidance.

	Year 1	Year 2	Year 3	Total Cost (£)
	1200	0	0	1200

Description: Interpretation Services
Type of cost: NHS
Please describe fully: Interpretation Services
Justification of Cost: Access to interpretation services/interpreters as/when required for public and patient advisory group members. Average costs £140 for 2-3hours. (3 x public and patient advisory group meetings across the fellowship at 60mins/meeting)

	Year 1	Year 2	Year 3	Total Cost (£)
	70	70	70	210

Description: Public and Patient Advisory Group - Meetings - Room booking and catering
Type of cost: NHS
Please describe fully: Public and Patient Advisory Group - Meetings - Room booking and catering
Justification of Cost: To have the option to book a room and provide catering for the 3 x one hour planned meetings during the fellowship for public and patient advisory group meetings. To be able to offer online/face to face/hybrid meetings to ensure representation and accessibility. Costings have been reviewed in conjunction with NHIR INVOLVE guidance

	Year 1	Year 2	Year 3	Total Cost (£)
	150	150	150	450

	Year 1	Year 2	Year 3	Total Cost (£)
Patient and Public Involvement Total	2170	970	970	4110

Other Direct Costs

Description: Stakeholder Workshop - Room Booking and Catering
Type of cost: NHS
Please describe fully: Stakeholder Workshop - Room Booking and Catering
Justification of Cost: To be able to book a hybrid room (to allow for both face to face and online participation) for the stakeholder workshop to develop the proposed co-produced tool and provide refreshments to attendees

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	500	500

Description: Stakeholder Workshop - Attendees Expenses
Type of cost: NHS
Please describe fully: Stakeholder Workshop - Attendees Expenses
Justification of Cost: To provide up to a £150 allowance per participant (15 participants proposed) to cover expenses (travel/carers costs) to attend the stakeholder workshop in person or an allowance to cover internet access/carers costs to be able to attend online

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	2250	2250

Description: Transcription from audio to text
Type of cost: NHS
Please describe fully: Transcription from audio to text
Justification of Cost: To transcribe audio recordings from interviews and focus groups into text ahead of data

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analysis (2 x focus groups of 60minutes in length = 120minutes of data to transcribe, 30 x 1:1 interviews of 120minutes in length = 3600minutes of data to transcribe). Average cost of transcription is £1.50/minute

	Year 1	Year 2	Year 3	Total Cost (£)
	0	5500	0	5500

Description: Interpretation Services
Type of cost: NHS
Please describe fully: Interpretation Services
Justification of Cost: Access to interpretation services/interpreters as/when required for service-user participation in focus groups and stakeholder workshop. Average costs £140 for 2-3hours. (2 x focus groups 60minutes and stakeholder workshop 3hours)

	Year 1	Year 2	Year 3	Total Cost (£)
	0	280	140	420

Description: MS Teams/Zoom License
Type of cost: NHS
Please describe fully: MS Teams/Zoom License
Justification of Cost: To be able to provide a hybrid model to interviews, focus groups, meetings and stakeholder workshop for widespread representation, participation and accessibility. To reduce costs where able, I will initially seek use through the HEI. The below costings are provided for use of Zoom Pro license if the HEI is unable to provide access. This programme allows increased time length of meetings.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	120	120	240

Description: Focus Groups - Attendees Expenses
Type of cost: NHS
Please describe fully: Focus Groups - Attendees Expenses
Justification of Cost: To provide up to a £150 allowance per participant (12 service-user participants and 8 NHS employees participants proposed) to cover expenses (travel/carers costs) to attend the focus groups in person or an allowance to cover internet access/carers costs to be able to attend online.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	3000	0	3000

Description: Focus Groups - Room booking and Catering
Type of cost: NHS
Please describe fully: Focus Groups - Room booking and Catering
Justification of Cost: To be able to provide a room with hybrid (face to face and online) capacity for 2 x focus groups and refreshments for participants

	Year 1	Year 2	Year 3	Total Cost (£)
	0	500	0	500

Description: SOECAT - Archiving Fee
Type of cost: NHS
Please describe fully: SOECAT - Archiving Fee
Justification of Cost: SOECAT completed in conjunction with JRES governance manager. Archiving fee provides the research cost included for the preparation and indefinite storage required secondary due to data collected for this study.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	601	0	601

	Year 1	Year 2	Year 3	Total Cost (£)
Other Direct Costs Total	0	10001	3010	13011

Training And Development

Pre-submission

Type of cost: NHS
Description: Short Courses
Item Description: University of Bristol Medical School - Introduction to Systematic Reviews and Meta-Analysis
Justification of Cost: To develop the knowledge and skills required to undertake a structured, comprehensive systematic review and meta-analysis within year one of the training programme to meet the aims/objectives of work package one. Online course with experts in systematic review and meta-analysis, recommended by RSS. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	880	0	0	880

Type of cost: NHS
Description: Short Courses
Item Description: NIHR ARC South London - King's Improvement Science introduces: the value of partnering with the public to improve health services
Justification of Cost: To increase understanding on PPIE and how PPIE has made a difference to the way that health research is conducted and delivered. Opportunity to attend a free short course within the locality of both partner and host organisation and thus accessible. Learning objectives compliment my learning needs within this area and will provide me the foundation knowledge and skills for this fellowship and as a platform to any training required in the future.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: Public Involvement Training
Justification of Cost: To increase understanding on how patients and the public can be involved across the research cycle. Costs for this course will support receival of certification of completion for my personal portfolio. Online course recommended by RSS. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	35	0	0	35

Type of cost: NHS
Description: Short Courses
Item Description: University of Oxford Introduction to Qualitative Research Methods
Justification of Cost: 5 day course delivered by qualitative experts, to introduce to a broad range of skills in qualitative research. To develop the knowledge and skills to prepare for collecting and analysing qualitative data, and understanding key philosophies and social theories. Online course with experts in qualitative research, recommended by RSS. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	1925	0	0	1925

Type of cost: NHS
Description: Short Courses
Item Description: University of Oxford Introduction to doing qualitative interviews
Justification of Cost: To gain the practical skills to undertake qualitative interviewing skills. To gain feedback on interviewing techniques. Online course with experts in qualitative methods, recommended by RSS. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	528	0	0	528

Type of cost: NHS
Description: Short Courses
Item Description: University of Oxford Introduction to analysing qualitative data

Pre-submission

Justification of Cost: To understand the principles and undertake practise of qualitative data analysis, including thematic analysis. This includes organising data and developing analysis to translate to a thesis chapter. Online course with experts in qualitative data analysis recommended by RSS. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	825	0	0	825

Type of cost: NHS
Description: Short Courses
Item Description: Social Research Association Introduction to Focus Groups
Justification of Cost: To develop the skills in focus group design and questioning. To develop moderation and facilitation skills, strategies to generate participative approach and overcome any challenges. Online course with experts in design, moderation and managing focus groups, recommended by RSS. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	235	0	0	235

Type of cost: NHS
Description: Short Courses
Item Description: Social Research Association Reflexivity in Qualitative Research
Justification of Cost: To gain the knowledge and skills in exploring the role of the researcher, their emotions within the process of qualitative research and develop practical strategies for practising reflexivity. Online course with experts in reflexivity, recommended by RSS. Course content meets my current learning needs having had nil previous training on reflexivity and is essential for this study. Online learning benefits my learning style and is easily accessible.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	220	0	220

Type of cost: NHS
Description: Short Courses
Item Description: City St George's, University of London - Introductory and Intermediary courses in NVivo for qualitative and mixed-methods analysis
Justification of Cost: To gain the skills and knowledge in methodological and analytical principles, overview of the software, analytic strategies for appropriate use of the software, step-by-step operation on software use and opportunity to have individual support with my project. An in-person course, which is provided by the HEI I will be enrolled in and therefore easily accessible and meets my learning needs. Advantages of being face to face include ability to undertake practical skills on utilising the software.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: The King's Fund Emerging Clinical Leaders Programme
Justification of Cost: To gain the required skills and develop as a senior leader, including building resilience and responsiveness to change. To create appropriate culture, strong leadership with patient-centred care, lead through change and build collaborations with other leaders. Online nature of the course and extended duration over four months compliments both my learning needs and opportunity to translate and mobilise skills/knowledge learnt back into my practice. This programme will enhance leadership training previously accessed through my NHS trust and is a widely recognised course within NHS leadership. Attendance will be reflected through demonstration in practice and within future employment opportunities.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	2260	0	2260

Type of cost: NHS
Description: Short Courses
Item Description: Point of Care Foundation Experience Based Co-Design

Pre-submission

Justification of Cost: To gain the skills required ahead of the stakeholder workshop in patient-centred service design techniques. Opportunity to attend this course in person will facilitate practical skills demonstration and compliment my prior experience in co-design through my CLAHRC NWL role. This course comes recommended from the RSS as highly recommended for learning co-design principles and implementing into practice.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	395	395

Type of cost: NHS
Description: Short Courses
Item Description: NIHR Introduction to knowledge mobilisation webinar
Justification of Cost: Gain the knowledge and skills for effective and impactful dissemination. Online course with NIHR experts in knowledge mobilisation; a key skill required for disseminating widely. Course content meets my current learning needs and will generate knowledge for future application beyond this fellowship. Online learning benefits my learning style and is easily accessible. Opportunity to meet and learn from individuals within NIHR.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: NIHR Engaging with policy makers: can your research influence policy webinar
Justification of Cost: Gain the knowledge and skills for effective and impactful dissemination. This course is particularly key given outcome from this study is to collaborate with policy makers and registering bodies regarding the evaluation tool. This course is free and online, providing great accessibility and is beneficial to successful achievement of the proposed outputs.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: NIHR Mobilising research knowledge with members of the public: the challenges and opportunities webinar
Justification of Cost: Gain the knowledge and skills for effective and impactful dissemination. This free online course will provide essential skills to moving the proposed outputs from this study into practice with engagement of key stakeholders and to members of the public who access UEC services including working in collaboration with PAG to ensure dissemination is understood and meaningful to all.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: The Health Foundation: Communicating your research
Justification of Cost: Gain the knowledge and skills for effective and impactful dissemination and communication of research. Opportunity to attend this free online course to aid skill development in innovative ways to communicate research. Skills/knowledge gained here will be utilised for an introductory session with PAG members, alongside signposting to PAG members for their attendance also if they wish.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: ARC South London PhD Writing Course
Justification of Cost: To further understand and develop personal writing style, refining thesis structure, opportunity for 1:1 feedback on writing and gain tips on editing large volumes of writing. Accessibility and locality to partner and host organisation makes this course an ideal fit with my learning and development, with opportunity to network with others in the sector and build potential collaborations for the future. Writing is a skill, which I would highlight as a weakness in regards to my style and structure. Therefore this course would be advantageous with addressing this and to be able to focus on specific thesis chapters for 1:1 feedback.

Pre-submission

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: HEI
Description: Applicant PhD Tuition Fees
Item Description: City St George's PhD/MPhil Health Services Research
Justification of Cost: Enrolment to City St George's PhD/MPhil programme, to work closely with primary doctoral supervisor throughout the programme, access to other learning modules/training the HEI has to offer. On review of the programme content, this compliments my research project and locality links closely with my employer and one of my academic supervisors, in which joint meetings have already commenced in preparation for this fellowship application. In addition, I have prior experience of studying at this HEI and I am familiar with methods in which content is provided and student platform.

	Year 1	Year 2	Year 3	Total Cost (£)
	5500	5665	5835	17000

Type of cost: NHS
Description: Workshops
Item Description: NIHR Advanced Fellowship/Development and Skills Enhancement Award Webinar/Fellowship Information Day
Justification of Cost: To understand opportunities post DCAF to continue clinical-academic career development and research leadership. This will compliment my career trajectory and ability to take the outcomes of this study further beyond the DCAF, alongside research generation in the future.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: City St George's Academic Skills Study Days
Justification of Cost: As part of PhD enrolment to City St George's to attend the one-day academic skills study days provided over 14weeks which includes introductory sessions to research methods, data analysis and scientific writing. Having previously studied at this HEI, the content provided and teaching methods are familiar to me. There is opportunity to network with others on similar programmes and career trajectory.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: NIHR and RSS Short Courses and Webinars
Justification of Cost: To attend the free online short courses and webinars provided by the NIHR and RSS relevant to my research study and personal development.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

Type of cost: NHS
Description: Short Courses
Item Description: Advanced Physiotherapy Practice Network online courses and webinars
Justification of Cost: To attend free online courses and webinars provided by APPN relevant to my clinical practice and research area to enhance my skills and development. There is opportunity to network with others on similar programmes and career trajectory.

	Year 1	Year 2	Year 3	Total Cost (£)
	0	0	0	0

	Year 1	Year 2	Year 3	Total Cost (£)
Training And Development Total	9928	8145	6230	24303

Pre-submission

Total Direct Costs

	Year 1	Year 2	Year 3	Total Cost (£)
Total Direct Costs	96690	105909	99875	302474

Indirect costs

HEI Indirect costs
No costs have been added

Commercial Indirect Costs
No costs have been added

Other Partner Organisation Indirect costs
No costs have been added

	Year 1	Year 2	Year 3	Total Cost (£)
Total Indirect Costs	0	0	0	0

Research Costs Requested From Funder

	Direct costs	Indirect costs/Overheads	Amount requested
Total Higher Education Institution Costs (80%)	0	0	0
Total Higher Education Institution Costs (100%)s	17000	0	17000
Total Commercial Costs	0	0	0
Total Other Parther Organisation Costs	0	0	0
Total NHS Research Costs	285474	0	285474
Total Research Costs Requested From Funder	302474	0	302474

Summary

	Year 1	Year 2	Year 3	Total Cost (£)
Salaries	**81990**	**85903**	**87206**	**255099**
Travel-subsistence-and-conference-fees	**1432**	**500**	**2029**	**3961**
Equipment	**1120**	**0**	**0**	**1120**
Consumables	**50**	**390**	**430**	**870**
Patient-and-Public-Involvement	**2170**	**970**	**970**	**4110**
Other-Direct-Costs	**0**	**10001**	**3010**	**13011**
Training And	**9928**	**8145**	**6230**	**24303**

Pre-submission

Development				
HEI-Indirect-costs	**0**	**0**	**0**	**0**
Commercial-Indirect-Costs	**0**	**0**	**0**	**0**
Other-Partner-Organisation-Indirect-costs	**0**	**0**	**0**	**0**
Total	**96690**	**105909**	**99875**	**302474**

NHS Support Costs

Cost Item:	Year 1	Year 2	Year 3	Total Cost (£)
SOECAT - NHS Support Costs	0	3188	0	3188.0

Justification:
SOECAT - NHS Support Costs : SOECAT completed in conjunction with JRES research governance lead. Costings below are calculated for informed consent, database search and eligibility criteria for participant recruitment for the study which will be undertaken by the lead applicant utilising the recruitment process outlined in the project plan. Nil funding for other sites is required as they will be approached for advertisement of the study only and will not be asked to actively recruit participants.

	Year 1	Year 2	Year 3	Total Cost (£)
NHS Support Costs Total	0	3188	0	3188

Have you discussed and agreed these costs with the Lead Network?	Yes

NHS Treatment Costs
No costs have been added

Have you discussed and agreed the NHS costs with the Lead Trust?	Yes

Is the patient/service user care being provided different from the usual standard treatment for the condition?
No

Usual Treatment Costs
No costs have been added

Excess Treatment costs/(savings)

	Cost per patient(£)	Year 1	Year 2	Year 3	Total Cost (£)
Total NHS Excess Treatment costs/(savings)	0	0	0	0	0

	Total (£)
NHS Support Costs requested from networks	3188
NHS Excess Treatment costs/savings	0
TOTAL NHS SUPPORT AND TREATMENT COSTS	3188

Pre-submission

Total Funding Required (including non-research costs)

	Total (£)
Total Research Costs Requested from Funder	302474
Total NHS (Support and Excess Treatment) Costs	3188
Total Funding Required	305662

Pre-submission

10. Uploads

Please upload the following documents, if necessary.

When uploading a document to your application, please ensure that it is in an appropriate file format. It is not possible to upload JPEG images, and with the exception of the SoECAT Form, which should be uploaded as an Excel spreadsheet, all files should be uploaded in Adobe PDF or Word DOC/DOCX format. Please refer to the Guidance Notes for further information on the attachments required.

(References - References.pdf) is included as an appendix within this file.

(Figures / Tables - Work Packages Graphic.pdf) is included as an appendix within this file.

(Research Timetable - Research Timetable - GANTT Chart.pdf) is included as an appendix within this file.

(SoECAT Template - SoECAT - Funder Export.xlsx) is included as an appendix within this file.

Pre-submission

11. Participants and Signatories

Participants
You are required to supply the name and email address (if not already registered on the ARAMIS application system) of each required participant. Everyone included in this section will be acting as a 'participant' in your application and must agree to be named as such.

By confirming participation, participants are acknowledging their involvement and input into this application, and are agreeing to be involved, as indicated, in any resultant fellowship.

Once the 'Administrative Authority or Finance Officer' participant has confirmed their participation, they will be able to access the 'Detailed Budget' section of the application form. You should collaborate with your 'Administrative Authority or Finance Officer participant to ensure that this section is completed satisfactorily.

You must ensure that all participants are happy for your application to be submitted before you submit it.

It is permissible for a single individual to serve as both a signatory and a participant, but they will need to be added separately under each heading.

Named participants must confirm their participation on your application before you will be able to select the 'SUBMIT' option. Please refer to the Submission Process Flow Diagram and the Applicant Guidance Notes for further information.

Pre-submission

Primary Doctoral Supervisor

Doctoral Primary Supervisor Participant 1

Title	Dr
First Name	Raheelah
Last Name	Ahmad
Position	Reader, Health Systems

Additional Academic Supervisor(s)

Supervisor Participant 1

Title	Professor
First Name	Robert
Last Name	Crouch
Position	Consultant Nurse

Supervisor Participant 2

Title	Professor
First Name	Heather
Last Name	Jarman
Position	Group Director for Health and Care Professions

Research

Practice Supervisor(s)

Practice Support Participant 1

Title	Dr
First Name	Edward
Last Name	Baker
Position	Consultant Nurse Emergency Care

Practice Support Participant 2

Title	Dr
First Name	Tim
Last Name	Noblet
Position	Consultant

Administrative Authority or Finance Officer

Administrative Authority or FO Participant

Title	Mr
First Name	Adam
Last Name	Bales
Position	Research Funding Manager (Pre-Award)

Pre-submission

Signatories
You are required to supply the name and email address (if not already registered on the ARAMIS application system) of each required 'Head of Department / Senior Manager' signatory. Once their contact details have been entered below, these signatories can be invited to log in and confirm their participation in your application.

Once the nominated signatories have confirmed their participation, they can access and complete the required 'Host Organisation Statement of Support' in the 'Training & Development and Research Support' section of the application form.

Your signatories must confirm their participation in your application and complete the section they are responsible for before you can select the 'submit' option.

Signatories must also approve your application after you select the 'submit' option but BEFORE the application submission deadline.

Signatory approval from your 'Head of Department / Senior Manager' signatories will result in the application being fully submitted to the NIHR. All parties (applicant, participants and signatories) will be notified of this via an automated system-generated email.

The NIHR will not accept your application unless fully approved by your signatories before the 1pm deadline on Thursday 14th November 2024.

Heads of Departments / Senior Managers

Head of Department or Senior Manager 1

Title	Professor
First Name	Steve
Last Name	Gillard
Position	Professor, Mental Health Research

Head of Department or Senior Manager 2

Title	Professor
First Name	Heather
Last Name	Jarman
Position	Group Director for Health and Care Professions Research

Pre-submission

12. Acknowledgement, Review and Submit

Conflict checks: Please declare any conflicts or potential conflicts of interest that you may have in undertaking this research, including any relevant, non-personal & commercial interest that could be perceived as a conflict of interest.
Nil conflicts or potential conflicts of interest

By completing this form, the programme reserves the right to share, in confidence, details of your application with other parts of the NIHR and with the Department of Health and Social Care.

Agreement to Terms and Conditions: I confirm that I have secured all necessary licences and ethical approvals in relation to the research and will abide by the terms of those licences and approvals in carrying out the research. I confirm that I have read the NIHR Carbon Reduction Guidelines and, where possible, have taken steps to reduce the carbon emissions generated by this research. I confirm that all material within this application, with the exception of sections that other participants are required to complete, is my own work and I have taken steps to ensure that no parts of the application have been inappropriately plagiarised. I undertake to conduct the research according to such guidelines for good research practice as the Host Organisation may from time to time lay down according to the NIHR research governance framework. I undertake to conduct the research according to such guidelines for good research practice as the Host Organisation may from time to time laydown according to the UK Policy Framework for Health and Social Care Research. In completing this, I declare that, to the best of my knowledge, the information provided in this application, is true, accurate and complete. I have read the Guidance Notes and agree to accept the process by which an application is assessed and agree to abide by the conditions under which an award may be granted.
☒Confirmed

Pre-submission

Appendices

1. References.pdf
2. Work Packages Graphic.pdf
3. Research Timetable - GANTT Chart.pdf
4. SoECAT - Funder Export.xlsx

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Appendix #1: References.pdf

Pre-submission

Braun & Clarke (2022) Thematic Analysis Thematic Analysis | SAGE Publications Ltd, **Cancelliere, C et al (2020) 'Risk of bias assessment according to study design' *BMJ Open*,** Carlsen, B. Glenton, C (2011) 'What about N? A methodological study of sample-size reporting in focus group studies' *BMC Medical Research Methodology* 11(26), **Chan, D et al (2014) 'A qualitative study on the roles and responsibilities of nurse consultants in Hong Kong' *International journal of nursing practice* (20),** Charters, S et al (2005) 'Learning from the past to inform the future – a survey of consultant nurses in emergency care' *Accident and Emergency Nursing,* **Critical Appraisal Skills Programme CASP Checklists - Critical Appraisal Skills Programme (casp-uk.net),** Crouch, R. 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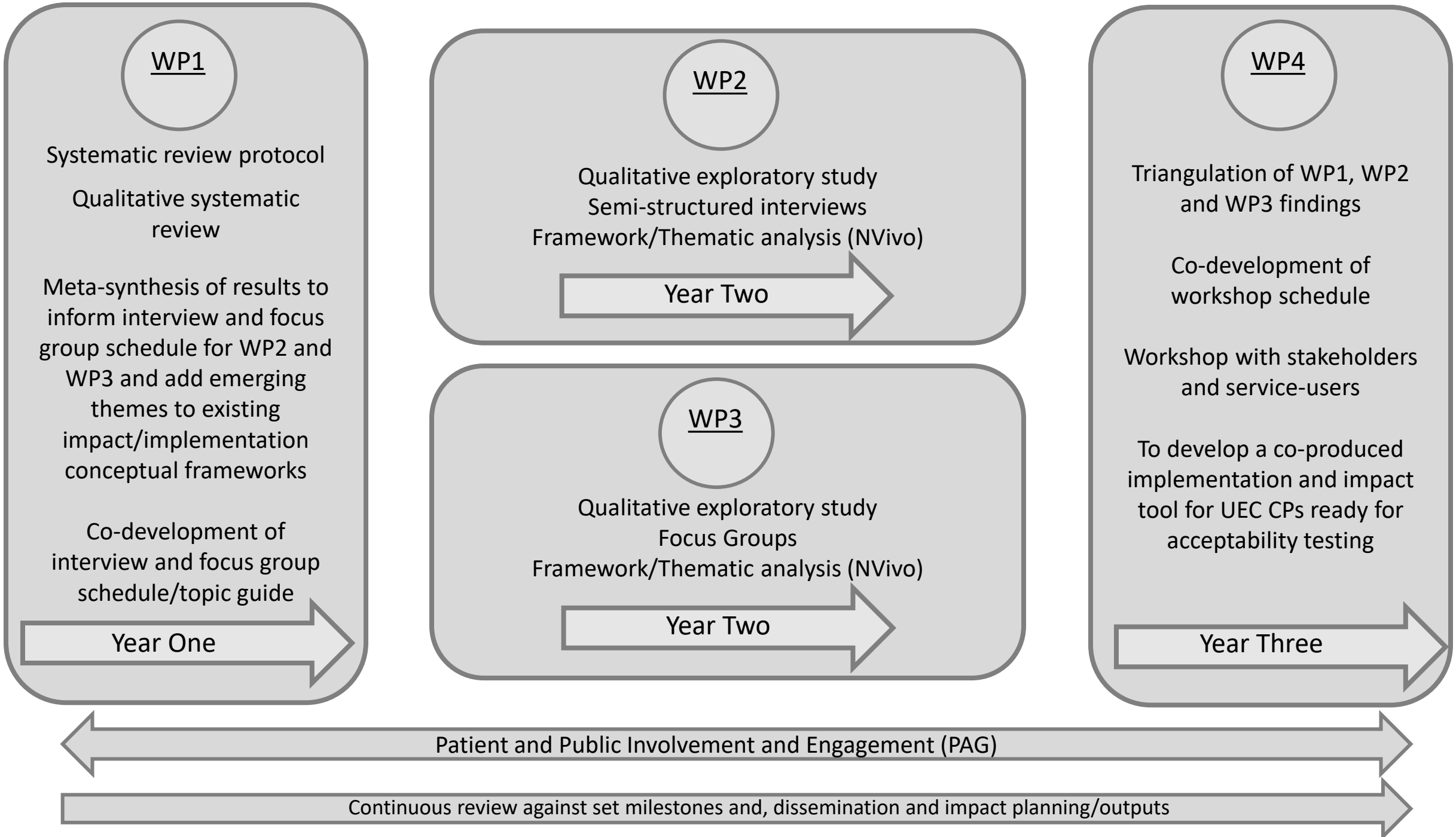
Pre-submission

Appendix #2: Work Packages Graphic.pdf

Pre-submission


WP1
Systematic review protocol
Qualitative systematic review
Meta-synthesis of results to inform interview and focus group schedule for WP2 and WP3 and add emerging themes to existing impact/implementation conceptual frameworks
Co-development of interview and focus group schedule/topic guide
Year One
WP2
Qualitative exploratory study
Semi-structured interviews
Framework/Thematic analysis (NVivo)
Year Two
WP3
Qualitative exploratory study
Focus Groups
Framework/Thematic analysis (NVivo)
Year Two
WP4
Triangulation of WP1, WP2 and WP3 findings
Co-development of workshop schedule
Workshop with stakeholders and service-users
To develop a co-produced implementation and impact tool for UEC CPs ready for acceptability testing
Year Three
Patient and Public Involvement and Engagement (PAG)
Continuous review against set milestones and, dissemination and impact planning/outputs

Appendix #3: Research Timetable - GANTT Chart.pdf

Pre-submission

Research Timetable – GANTT Chart

Tasks/Milestones		Year One (Commencing Sept 2025)				Year Two (Commencing Sept 2026)				Year Three (Commencing Sept 2027)			
	Pre-fellowship	Month 1-3	Month 4-6	Month 7-9	Month 10-12	Month 1-3	Month 4-6	Month 7-9	Month 10-12	Month 1-3	Month 4-6	Month 7-9	Month 10-12
Research Management													
Project management meetings including finance meetings													
Academic Supervision													
Clinical Supervision													
Research Planning													
Ethics applications for WP2 and WP3													
Database and resource development													
Systematic review protocol (WP1)													
Semi-structured interviews protocol (WP2)													
Focus groups protocol (WP3)													
Project Completion													
WP1 Qualitative systematic review completion													
WP1 Qualitative systematic review data synthesis													
WP2 Recruitment for interviews													
WP2 Semi-structured interview completion, data collection													
WP2 Data analysis													
WP3 Recruitment for focus groups													
WP3 Focus groups completion, data collection													
WP3 Data analysis													
WP4 - WP2 and WP3 Triangulation of data													
WP4 Evaluative tool development													
WP4 Stakeholder workshop													
PPIE													
Recruitment to PAG													
PAG group meetings													
Update bulletins sent to PAG members													
PAG group training activities													
Involvement in WP4 stakeholder workshop development													
Involvement in research cycle milestones													
Training and Development													
Research Visits													
Research Training/Courses													
Clinical Training/Courses													
Leadership Training/Courses													
Conferences													
Impact Planning and Dissemination/Outputs													
Posters for conferences													
WP1 Systematic review write-up and submission for publication													
WP2/3 Interview and focus group results write-up and submission for publication													
Thesis write-up													
Applications for future funding/fellowships													
WP4 co-produced evaluative tool write-up													

Appendix #4: SoECAT - Funder Export.xlsx

Pre-submission